



Community views of neurodiversity, models of disability and autism intervention: Mixed methods reveal shared goals and key tensions

Patrick Dwyer^{1,2}, Ava N Gurba^{3,4*}, Steven K Kapp^{5*}, Elizabeth Kilgallon^{6,7*}, Lynnette H Hersh^{1,8}, David S Chang¹, Susan M Rivera^{1,9†} and Kristen Gillespie-Lynch^{10†}

Abstract

Controversies regarding the neurodiversity movement may be exacerbated by confusion over its meaning. For example, some suggest neurodiversity entails acceptance of the social model, whereas others describe it as more nuanced. We aimed to help resolve conflicting viewpoints by inviting insights from 504 autistic and autism community members (278 autistic, 226 non-autistic), including 100 researchers (41 autistic), 122 professionals (35 autistic) and 162 parents/caregivers (53 autistic). They rated the neurodiversity movement, social model, medical model and intervention goals, and answered open-ended questions regarding the meaning of neurodiversity, the neurodiversity movement and disability models. Neurodiversity movement support was associated with endorsing societal reform and making environments more supportive, and lower support for normalization and adaptive skill interventions, though teaching adaptive skills was widely supported overall. Although participants often suggested the social model attributes disability solely to society, this ‘strong’ view was not universal. ‘Strong’ social model supporters still endorsed some interventions targeting individuals’ characteristics (specifically, teaching adaptive skills, curing depression and epilepsy). Findings confirm that neurodiversity movement supporters denounce normalization, but are open to individualized supports. Findings highlight strong support for autistic leadership (especially among autistic people, including autistic parents) and for societal reform, and suggest that oversimplified rhetoric may cause confusion regarding advocates’ views.

Lay abstract

What is already known about the topic? Some people support the neurodiversity movement and other people criticize it. They often disagree about what the neurodiversity movement means. Confusion about what the neurodiversity movement is makes it hard for people to agree about how best to support autistic or disabled people. **What does this article add?** We studied autistic and autism community members’ views on the neurodiversity movement and autism interventions. Most participants supported the neurodiversity movement. Support for the neurodiversity movement was related to wanting to change society and make places more comfortable for autistic people. Neurodiversity supporters felt autistic children should choose their own intervention goals and that autistic adults should help children choose goals. Neurodiversity supporters did not think autistic people should be taught to act less autistic. Both those agreeing and disagreeing with the neurodiversity movement said that it is important to help autistic people be well. Many participants said that society causes all the difficulties disabled people experience. Yet they were somewhat open to helping people grow by teaching them useful skills and trying to cure depression and epilepsy.

¹University of California, Davis, USA

²La Trobe University, Australia

³Stony Brook University, USA

⁴Drexel University, USA

⁵University of Portsmouth, UK

⁶Yale University, USA

⁷University of Massachusetts Boston, USA

⁸Alliant International University, USA

⁹University of Maryland, USA

¹⁰College of Staten Island & The Graduate Center, City University of New York, USA

*Co-second authors.

†Co-senior authors.

Corresponding author:

Patrick Dwyer, Olga Tennison Autism Research Centre, School of Psychology and Public Health, La Trobe University, Bundoora, VIC, 3086, Australia.
Email: patrick.dwyer@latrobe.edu.au



社区对神经多样性、残疾模式与孤独症干预的看法：混合方法揭示共同目标与关键张力

Patrick Dwyer^{1,2}, Ava N Gurba^{3,4*}, Steven K Kapp^{5*}, Elizabeth Kilgallon^{6,7*}, Lynnette H Hersh^{1,8}, David S Chang¹, Susan M Rivera^{1,9†} 和 Kristen Gillespie-Lynch^{10†}

摘要

关于神经多样性运动的争议，可能因其含义的混淆而加剧。例如，有人认为神经多样性意味着接受社会模式，而另一些人则将其描述为更为微妙的概念。我们旨在通过邀请504名孤独谱系及孤独谱系社群成员（278名孤独谱系，226名非孤独谱系）——包括100名研究人员（41名孤独谱系）、122名专业人员（35名孤独谱系）和162名家长/照护者（53名孤独谱系）——提供见解，以帮助解决观点冲突。他们对神经多样性运动、社会模式、医学模式及干预目标进行了评分，并就神经多样性、神经多样性运动和残障模式的含义回答了开放式问题。对神经多样性运动的支持，与赞同社会改革、使环境更具支持性相关联，并且与对正常化和适应性技能干预的支持度较低相关，尽管总体上教授适应技能得到了广泛支持。尽管参与者常认为社会模式将残障完全归因于社会，但这种‘强硬’观点并非普遍存在。‘强硬’的社会模式支持者仍然赞同一些针对个体特征的干预（具体而言，教授适应技能、治愈抑郁和癫痫）。研究结果证实，神经多样性运动的支持者谴责正常化，但对个体化支持持开放态度。研究结果突显了对孤独谱系群体领导（尤其是在孤独谱系群体中，包括孤独谱系家长）和社会改革的强烈支持，并表明过于简化的修辞可能导致对倡导者观点的混淆。

简明摘要

关于该主题已知的信息有哪些？ 有些人支持神经多样性运动，另一些人则批评它。他们对于神经多样性运动的含义常常存在分歧。对神经多样性运动是什么的困惑，使得人们难以就如何最好地支持孤独谱系或残障群体达成一致。**本文补充了什么？** 我们研究了孤独谱系群体及孤独谱系社群成员对神经多样性运动和孤独症干预的看法。大多数参与者支持神经多样性运动。对神经多样性运动的支持与希望改变社会、为孤独谱系群体创造更舒适的环境相关。神经多样性支持者认为，孤独谱系儿童应自行选择其干预目标，且成年孤独谱系个体应帮助儿童选择目标。神经多样性支持者不认为应该教导孤独谱系群体减少表现出孤独谱系特质。无论赞同还是反对神经多样性运动的人士都表示，帮助孤独谱系群体保持良好状态很重要。许多参与者认为，社会造成了残障人士所经历的所有困难。然而，他们对于通过教授有用技能以及尝试治愈抑郁和癫痫来帮助人们成长，持一定开放态度。

加州大学戴维斯分校，美国²拉筹伯大学，澳大利亚³石溪大学，美国⁴德雷塞尔大学，美国⁵朴茨茅斯大学，英国⁶耶鲁大学，美国⁷马萨诸塞大学波士顿分校，美国⁸联合国国际大学，美国⁹马里兰大学，美国

¹⁰ 史泰登岛学院 & 纽约市立大学研究生院
美国纽约

*共同第二作者。

†共同资深作者。

通讯作者： Patrick Dwyer, Olga Tennison自闭症研究中心，心理与公共卫生学院，拉筹伯大学，Bundoora, VIC, 3086, 澳大利亚。

电子邮件: patrick.dwyer@latrobe.edu.au

Implications for practice, research or policy. To better help autistic people, we should listen to participants in this study. They called for improving society, building more spaces where autistic people feel comfortable and helping autistic people lead change. They warned against teaching autistic people to hide their autism. Critics of the neurodiversity movement should read this article to better understand what neurodiversity movement supporters believe. By taking time to learn from one another, we can work together to better support autistic people.

Keywords

autism, intervention goals, neurodiversity, normalization, social model

Introduction

The neurodiversity movement (NDM), and its impacts on supports for and research about neurodivergent people, is intensely controversial (Bailin, 2019; Baron-Cohen, 2019; Chellappa, 2024; Lerner et al., 2023). Many autistic (Robertson, 2010; Walker & Raymaker, 2020) and non-autistic people (Des Roches Rosa, 2019; Savarese et al., 2010) speak strongly in support of approaches inspired by the NDM, while other people (Costandi, 2019; Hiari, 2018; Lutz, 2015) strongly denounce them.

Unfortunately, the concept of neurodiversity can be ambiguous, complicating debates regarding the NDM. One meaning of ‘neurodiversity’ is purely descriptive – referring to the reality of human neurocognitive diversity (Singer, n.d.; Walker, 2021) – but some understand it to be prescriptive or ideological (Ne’ eman & Pellicano, 2022). It is also the basis of neurodiversity approach(es) (Dwyer, 2022), alternately called a ‘perspective’ (Robertson, 2010), ‘paradigm’ (Walker, 2014, 2021) or ‘framework’ (Kapp, 2020), that are clearly prescriptive.¹ The NDM champions neurological diversity and the neurodiversity approaches. But what, precisely, does this mean? Not only is neurodiversity often simply misunderstood (den Houting, 2019), but various parties can hold different legitimate views of what neurodiversity means (Chapman, 2020a; Dwyer, 2022; Ne’ eman & Pellicano, 2022), in part because the neurodiversity concept was not developed by any single person (Botha et al., 2024; Dekker, 2023). This confusion makes it harder to resolve debates regarding the appropriateness of neurodiversity-affirming approaches towards autistic and neurodivergent people, their needs and intervention/supports.

Disability, Neurodiversity and Intervention

One particularly important question is the status of disability in neurodiversity approaches. While numerous NDM supporters strenuously deny claims that the NDM rejects the reality of disability (Ballou, 2018; den Houting, 2019; Thinking Person’s Guide to Autism, n.d.), the ways in which neurodiversity approaches conceptualize disability

remain somewhat contentious. Models of disability such as the medical model (MM) and social model (SM) pre-date the emergence of autistic/neurodiversity advocacy in the 1990s (Botha et al., 2024; Kapp, 2013; Pripas-Kapit, 2020). For much of the 20th century, researchers and clinicians have generally operated within the dominant MM of disability, which – at least in its conventional, ‘strong’ form – conceptualizes autism and other neurodevelopmental differences as disorders and deficits rooted in biological differences between people (Kapp, 2023). In the MM, these biological differences might be understood to have a genetic aetiology, environmental influences (e.g. exposure to substances *in utero*) might be recognized or a genetic-environmental interaction might be recognized, but either way, in the traditional MM, the internal biological differences directly lead to a person’s experience of disability-related challenges. Under this model, experts prescribed interventions to cure, prevent or suppress autism and autistic behaviours (Kapp, 2023). Admittedly, in practice, individuals and groups using pathologizing rhetoric consistent with the MM do often end up advocating for societal changes of one form or another (Baker, 2011), but these might raise rights and justice concerns (e.g. restraints and subminimum wage; National Council on Severe Autism, n.d.) and can be an adaptation to the infeasibility of curing or preventing many disabilities using currently available technologies (Lutz, 2024).

In contrast to the MM, some authoritative sources suggest that neurodiversity approaches are aligned with the SM of disability (Bölte et al., 2021; Labour Party Autism/Neurodiversity Manifesto Steering Group, 2018; Thinking Person’s Guide to Autism, n.d.), which – again at least in its conventional ‘strong’ form – attributes difficulties associated with disabilities solely to societal inequalities, including active discrimination against people with disabilities (Shakespeare, 2006; illustrated in Figure 1). In its traditional form, the SM separates disability from ‘impairments’, the actual biological differences between disabled and non-disabled people.² Instead of directly causing disability, a person’s biological differences would only give rise to disability if societal institutions, infrastructure and practices were exclusionary (Oliver, 1990, 2009). For

对实践、研究或政策的启示. 为了更好地帮助孤独谱系群体，我们应当倾听本研究参与者的意见。他们呼吁改善社会，建立更多让孤独谱系群体感到舒适的空间，并帮助孤独谱系群体引领变革。他们警告不要教导孤独谱系群体隐藏他们的孤独谱系特质。神经多样性运动的批评者应阅读本文，以更好地理解神经多样性运动支持者的信念。通过花时间相互学习，我们可以共同努力，更好地支持孤独谱系群体。

Key关键词

孤独谱系，干预目标，神经多样性，正常化，社会模式

引言

神经多样性运动（NDM）及其对神经殊异群体的支持与研究的影响，存在激烈的争议（Bailin, 2019; Baron-Cohen, 2019; Chellappa, 2024; Lerner et al., 2023）。许多孤独谱系（Robertson, 2010; Walker & Raymaker, 2020）和非孤独谱系人士（Des Roches Rosa, 2019; Savarese et al., 2010）强烈支持受NDM启发的做法，而另一些人（Costandi, 2019; Hiari, 2018; Lutz, 2015）则强烈谴责这些做法。

遗憾的是，神经多样性的概念可能具有模糊性，这使得围绕神经多样性模型的辩论变得复杂。“神经多样性”的一个含义纯粹是描述性的——指人类神经认知多样性的现实（Singer, 未注明日期; Walker, 2021）——但有些人将其理解为规范性或意识形态性的（Ne’ eman与Pellicano, 2022）。它也是神经多样性方法的基础（Dwyer, 2022），有时被称为“视角”（Robertson, 2010）、“范式”（Walker, 2014, 2021）或“框架”（Kapp, 2020），这些显然是规范性的。¹ 神经多样性模型倡导神经多样性以及神经多样性方法。但这究竟意味着什么？不仅神经多样性常常被简单地误解（den Houting, 2019），而且各方对神经多样性的含义可能持有不同的合理观点（Chapman, 2020a; Dwyer, 2022; Ne’ eman与Pellicano, 2022），部分原因在于神经多样性概念并非由任何单个人发展而来（Botha等人, 2024; Dekker, 2023）。这种混淆使得围绕针对孤独谱系和神经殊异群体、他们的需求以及干预/支持的神经多样性肯定方法的适当性的辩论更难解决。

残障、神经多样性与干预

一个特别重要的问题是残障在神经多样性方法中的地位。尽管许多NDM支持者极力否认NDM拒绝承认残障现实的说法（Ballou, 2018; den Houting, 2019; Thinking Person’s Guide to Autism, n.d.），但神经多样性方法对残障的概念化方式

仍然存在一些争议。诸如医学模式（MM）和社会模式（SM）等残疾模式的出现早于孤独谱系/神经多样性倡导在1990年代的兴起（Botha et al., 2024; Kapp, 2013; Pripas-Kapit, 2020）。在20世纪的大部分时间里，研究人员和临床医生通常都在主导的MM残疾模式下运作，该模式——至少在其传统的‘强’形式中——将孤独谱系及其他神经发育差异概念化为根植于人与人之间生物学差异的障碍和缺陷（Kapp, 2023）。在MM中，这些生物学差异可能被理解为具有遗传病因，环境影响因素（例如在子宫内接触某些物质）可能被承认，或者遗传-环境相互作用可能被承认，但无论如何，在传统的MM中，内在的生物学差异直接导致一个人经历与残障相关的挑战。在这种模式下，专家们会制定干预措施来治愈、预防或抑制孤独谱系及孤独谱系行为（Kapp, 2023）。诚然，在实践中，使用与MM一致的病理化话语的个人和团体最终往往倡导某种形式的社会变革（Baker, 2011），但这些变革可能引发权利和正义方面的担忧（例如约束措施和低于最低工资; National Council on Severe Autism, n.d.），并且可能是对使用当前可用技术治愈或预防许多残障不可行性的适应（Lutz, 2024）。

与医学模型相反，一些权威资料表明神经多样性方法与残障的社会模型相一致（Bölte等人, 2021; 工党孤独谱系/神经多样性宣言指导小组, 2018; Thinking Person’s Guide to Autism, 未注明日期），该模型——至少在其传统的“强”形式中——将残障相关的困难完全归因于社会不平等，包括对残障人士的主动歧视（Shakespeare, 2006; 如图1所示）。在其传统形式中，社会模型将残障与“损伤”——即残障与非残障人士之间实际的生物学差异——区分开来。² 一个人的生物学差异并不会直接导致残障，只有当社会制度、基础设施和实践具有排斥性时，这些差异才会引发残障（Oliver, 1990, 2009）。

		Prescribed Actions towards Autism/Neurodevelopmental Difference/Disability			
		Status Quo	Problematic or Regressive Societal Change	Change to Individual	Meaningful, Progressive Societal Change
Description of Autism/Neurodevelopmental Difference	Affirming				Interactionist/ “Weak” Social Model-Aligned Neurodiversity Movement
		“Neurodiversity Lite”			“Strong” Social Model-Aligned Neurodiversity Movement
	“Super-powers”; Denies Disability				
	Pathologizing		Medical Model		

Figure 1. Illustration of how various models of and approaches towards autism/disability could characterize/understand autism and neurodevelopmental differences (in a pathologizing manner, an affirming manner or a manner that does not recognize challenges and focuses on ‘abilities’ and ‘superpowers’ only) and whether they prescribe changing the individual, society or both. Note that this table/figure is intended to approximately characterize models for illustrative purposes, and does not represent the full complexity of the models. For example, other dimensions may be relevant, such as the extent to which individuals’ autonomy is facilitated or suppressed, the extent to which models are reductionist or embrace complexity, whose priorities are most valued, or the extent to which disability is understood to be internal to the individual versus socially caused (closely related but not necessarily identical to the dimension of prescribed actions towards autism, difference and disability). Furthermore, it is important to note that maintenance of the status quo – in the sense of not prescribing interventions aiming to change individuals or society – could still lead to individuals attempting to mask/camouflage their neurodivergent traits or perform according to ableist expectations. Thus, lack of formal prescribed intervention could still lead to the individual attempting to change themselves. In this regard, and in the compatibility of ‘neurodiversity lite’ with attempts to change the individual, ‘neurodiversity lite’ can resemble the medical model with pathologizing language removed.

example, some autistic people, in some contexts, can communicate and connect with other autistics as successfully as non-autistic people can with other non-autistics (Crompton, Ropar, et al., 2020; Crompton, Sharp, et al., 2020). This and other research examining the double empathy problem (Milton, 2012), suggests that autistic social disabilities are not, in fact, inherent deficits in autistic people and may often be caused by exclusionary behaviours from neurotypicals – just as the ‘strong’ SM claims. However, some NDM and/or disability³ advocates, including the first author, reject the ‘strong’ form of the SM (Ballou, 2018; Kapp, 2013; Shakespeare, 2014) and support various interactionist or social-relational models of disability (Autisticic, 2020; Chapman, 2021; Dwyer, 2022; Kapp, 2013; Shakespeare, 2014) generally conceptualizing disability as the product of poor person–environment fit: a mismatch between the person and their environment disables them. Neither the person nor the environment is necessarily always the sole culprit. As such, in different contexts, intervention might appropriately focus on either changing the person, changing their

environment or both (see Figure 1). While the NDM clearly suggests that intervening to cure autism would always be inappropriate, some advocates suggest that interventions could still teach useful adaptive skills (Autistic Self Advocacy Network, 2021; Dwyer, 2022; Kapp et al., 2013; Sinclair, 1992), seemingly in alignment with these interactionist views. In addition, not everyone endorsing ‘the social model’ supports a ‘strong’ SM. While some ‘strong’ SM supporters reject any possibility of common ground with the MM (Cameron, 2024), for decades, others have called for the SM to expand beyond its societal focus to acknowledge biological differences as important in themselves (Crow, 1996; B. Hughes & Paterson, 1997), creating what might be called a ‘weak’ SM (see Figure 1). Even Mike Oliver, a leading ‘strong’ SM champion, readily acknowledged that its dismissal of biological differences made it only a politically useful tool, not a complete theory (Oliver, 2009, 2013). Thus, contrary to some assertions (J. A. Hughes, 2021), some SM-endorsing neurodiversity advocates might support some individual-focused interventions.

		Prescribed Actions towards Autism/Neurodevelopmental Difference/Disability			
		Status Quo	Problematic or Regressive Societal Change	Change to Individual	Meaningful, Progressive Societal Change
Description of Autism/Neurodevelopmental Difference	Affirming				Interactionist/ “Weak” Social Model-Aligned Neurodiversity Movement
		“Neurodiversity Lite”			“Strong” Social Model-Aligned Neurodiversity Movement
	“Super-powers”; Denies Disability				
	Pathologizing		Medical Model		

图 1. 展示不同关于孤独谱系/残障的模型和路径如何（以病理化、肯定化或仅关注‘能力’与‘超能力’而不承认挑战的方式）描述/理解孤独谱系与神经发育差异，以及它们是否主张改变个体、社会或两者。请注意，此表格/图表旨在为说明目的大致描述模型，并不代表模型的全部复杂性。例如，其他维度可能相关，如个体自主性被促进或抑制的程度、模型是简化论还是拥抱复杂性、谁的优先事项最受重视，或残障被理解为个体内在还是社会造成（这与针对孤独谱系、差异和残障的处方行动维度密切相关但不完全相同）。此外，重要的是要注意，维持现状——即不主张旨在改变个体或社会的干预——仍可能导致个体试图掩饰其神经特殊特质或按照健全主义期望行事。因此，缺乏正式的处方干预仍可能导致个体试图改变自身。在这方面，以及‘神经多样性轻量版’与试图改变个体的兼容性上，‘神经多样性轻量版’可与去除病理化语言的医学模式相似。

例如，在某些情境下，一些孤独谱系群体能够与其他孤独谱系群体进行沟通和建立联系，其成功程度与非孤独谱系群体之间的互动相当 (Crompton, Ropar等人, 2020; Crompton, Sharp等人, 2020)。这项研究以及其他探讨双向同理心问题的研究 (Milton, 2012) 表明，孤独谱系群体的社交残障实际上并非他们固有的缺陷，而往往是由神经典型群体的排斥性行为所导致——正如“强”社会模型所主张的那样。然而，一些NDM及/或残障³ 倡导者，包括第一作者，拒绝社会模型的‘强’形式 (Ballou, 2018; Kapp, 2013; Shakespeare, 2014)，并支持各种互动主义或社会关系残疾模型 (Autisticic, 2020; Chapman, 2021; Dwyer, 2022; Kapp, 2013; Shakespeare, 2014)，通常将残障概念化为个人与环境适配不良的产物：个人与其环境之间的不匹配使他们致残。个人或环境都不必然是唯一的罪魁祸首。因此，在不同的情境下，干预可能适当地侧重于改变个人、改变其

环境或两者兼而有之（见图1）。虽然NDM明确表明干预以治愈孤独谱系始终是不恰当的，但一些倡导者认为干预仍可教授有用的适应技能（孤独谱系自我倡导网络, 2021; Dwyer, 2022; Kapp等, 2013; Sinclair, 1992），似乎与这些互动主义观点保持一致。此外，并非所有支持‘社会模式’的人都赞同一种‘强’社会模式。虽然一些‘强’社会模式的支持者拒绝任何与医学模型达成共识的可能性 (Cameron, 2024)，但几十年来，另一些人一直呼吁社会模式应超越其社会性关注，承认生物学差异本身的重要性 (Crow, 1996; B. Hughes & Paterson, 1997)，从而形成了一种可被称为‘弱’社会模式的观念 (见图1)。即使是‘强’社会模式的主要倡导者Mike Oliver也坦率承认，其对生物学差异的忽视使其仅成为一种政治上有用的工具，而非一个完整的理论 (Oliver, 2009, 2013)。因此，与某些断言 (J. A. Hughes, 2021) 相反，一些支持社会模式的神经多样性倡导者可能支持某些以个体为中心的干预。

Because autism interventions have generally focused on changing autistic people rather than society (Bottema-Beutel et al., 2018; Broderick & Roscigno, 2021; Scott et al., 2019), advocates might simply see the SM as a tool for shifting towards greater balance between these objectives, and modifying intervention goals and practices accordingly. Notably, neurodiversity advocates often acknowledge that some co-occurring conditions – for example, clinical anxiety, depression, epilepsy – should be medically cured if possible (Autistic Self Advocacy Network, 2022; Chapman, 2019; Kapp, 2020; Love, 2022; Thinking Person’s Guide to Autism, n.d.), though opinions vary (e.g. see Armstrong, 2010, a non-autistic writer). This leads to an important question – do autistic and autism community members who claim to support the SM endorse a ‘strong’ SM?

Further complicating the situation, not only is the NDM sometimes mischaracterized as less capable of recognizing and supporting personal difficulties than it often is (Bailin, 2019; Ballou, 2018), NDM supporters have expressed concern over ‘neurodiversity lite’ views: adoption of neurodiversity rhetoric and slogans without a full understanding of the NDM or commitment to its goals, and perhaps even to oppose the NDM’s goals or seek personal profit (Chapman, 2023; den Houting, 2019; Neumeier, 2018; Roberts, 2021). For example, ‘neurodiversity lite’ could involve performatively speaking about celebrating neuro-cognitive differences while simultaneously promoting masking/camouflaging and reinforcing neurotypical social norms, and/or stereotyping neurodivergent people as having marketable ‘super powers’ that can be exploited to increase productivity.

Thus, in summary, perspectives regarding the nature of disability among NDM advocates remain ambiguous, though many NDM advocates appear to have more nuanced perspectives than implied by a ‘strong’ SM.

This study

This study used a mixed-methods approach to explore perspectives regarding these issues in the autistic and autism communities. Importantly, we do not aim to suggest that particular views should be adopted merely because they are more or less popular, but to illuminate common understandings of the NDM and their implications for interventions and supports, so that discussions and debates can proceed with greater clarity and fewer misunderstandings. We were especially curious about – and sceptical of – whether neurodiversity advocates commonly endorse views consistent with a ‘strong’ SM: in particular, focusing solely on changing society and rejecting all efforts to change autistic people.

Through social media and requests for autism organizations to distribute recruitment information, participants who were autistic, or had a connection to autism in other ways

(such as being a family member of an autistic person, or having a professional/research role related to autism) were recruited and asked to quantitatively rate their support for the NDM, models of disability and intervention goals. Furthermore, to contextualize these data, participants were also asked to qualitatively define neurodiversity, the NDM and models of disability. We hypothesized that:

1. Support for the NDM would be associated with reduced endorsement of normalization/behaviour reduction goals, but not with altered endorsement of adaptive skill or well-being/mental health goals, in alignment with findings from a study examining attitudes towards neurodiversity and interventions in the autism community around a decade ago (Kapp et al., 2013).
2. Autistic neurodiversity proponents would show a greater focus on environmental and societal barriers, and support for interventions capable of changing these than non-autistic neurodiversity proponents, as they would be less susceptible to ‘neurodiversity lite’ than non-autistic neurodiversity proponents.
3. Many participants who report supporting the SM would endorse supports that aim to change the autistic person through teaching of adaptive skills, not just reforms to environments and society, inconsistent with the tenets of the ‘strong’ SM.

As such, this mixed-methods study relates to a number of domains of inquiry. It is inextricably concerned with disability studies and theory, while findings regarding the perceived desirability of specific intervention goals to participants and subsets of participants also have implications for practice. Parts of the article relate to philosophy, logical consistency and ontology. The study is empirical and an exercise in scientific hypothesis-testing, but findings also have political implications.

Methods

Participants

This study’s sample is also described in the study by Hersh et al. (2024). Participants were recruited from social media and through contacting autism organizations in multiple countries; to be eligible, they were required to be adults with a connection to autism, such as being autistic, being family members of an autistic person and/or autism professional/research roles. After removal of putatively invalid/bot/fake data and incomplete responses whose validity could not be evaluated (see Supplemental Appendix A; see also Goodrich et al., 2023; Pellicano et al., 2023), 504 responses (278 autistic, 226 non-autistic) remained (see Table 1). Most participants came from Canada (*n*=55), the United Kingdom (*n*=60) or the United States (*n*=337).

由于孤独症干预通常侧重于改变孤独谱系群体而非社会（Bottema-Beutel等, 2018; Broderick与Roscigno, 2021; Scott等, 2019），倡导者们可能仅仅将社会模式视为一种工具，用于推动在这些目标之间实现更好的平衡，并相应地调整干预目标 and 实践。值得注意的是，神经多样性倡导者常常承认，一些共病状况——例如临床焦虑、抑郁、癫痫——如果可能的话，应该通过医学手段治愈（孤独谱系自我倡导网络, 2022; Chapman, 2019; Kapp, 2020; Love, 2022; Thinking Person’s Guide to Autism, 无日期），尽管意见不尽相同（例如，参见非孤独谱系作家Armstrong, 2010的观点）。这就引出了一个重要问题——那些声称支持社会模式的孤独谱系及孤独谱系社群成员，他们赞同的是‘强’社会模式吗？

进一步使情况复杂化的是，不仅神经多样性模型有时被错误地描述为识别和支持个人困难的能力不如实际水平（Bailin, 2019; Ballou, 2018），神经多样性模型的支持者也对‘神经多样性轻量版’观点表示担忧：即采纳神经多样性修辞和口号，却未充分理解神经多样性模型或致力于其目标，甚至可能反对神经多样性模型的目标或寻求个人利益（Chapman, 2023; den Houting, 2019; Neumeier, 2018; Roberts, 2021）。例如，‘神经多样性轻量版’可能涉及表演性地谈论庆祝神经认知差异，同时却提倡掩饰/伪装并强化神经典型社会规范，以及/或将神经殊异群体刻板印象化为拥有可被利用以提升生产力的市场化‘超能力’。

因此，总结而言，NDM倡导者对残障本质的看法仍存在模糊之处，尽管许多NDM倡导者的观点似乎比‘强’SM所暗示的更为微妙。

本研究

本研究采用混合方法研究，探讨孤独谱系及孤独症社群对这些议题的看法。重要的是，我们并非旨在建议某些观点仅因其受欢迎程度高低而应被采纳，而是为了阐明对NDM的普遍理解及其对干预和支持的影响，从而使讨论和辩论能够更清晰、误解更少地进行。我们尤其好奇并怀疑——神经多样性倡导者是否普遍赞同与‘强’SM一致的观点：特别是，仅专注于改变社会，并拒绝所有改变孤独谱系群体的努力。

通过社交媒体以及向孤独谱系组织分发招募信息的请求，参与者——无论是孤独谱系人士，或是在其他方面与孤独谱系有关联的人员

（例如作为孤独谱系人士的家庭成员，或拥有与孤独谱系相关的专业/研究角色）——被招募并要求对其对神经多样性模型、残疾模式及干预目标的支持程度进行定量评分。此外，为了将这些数据置于具体情境中，参与者还被要求定性定义神经多样性、神经多样性模型及残疾模式。我们假设：

1. 对NDM的支持将与减少对正常化/行为减少目标的认同相关联，但不会与对适应技能或幸福感/心理健康目标的认同改变相关联，这与大约十年前一项研究孤独谱系社群对神经多样性和干预态度的发现相一致（Kapp et al., 2013）。
2. 孤独谱系神经多样性支持者会比非孤独谱系神经多样性支持者更关注环境和社会障碍，并更支持能够改变这些障碍的干预，因为他们比非孤独谱系神经多样性支持者更不易受‘神经多样性轻量版’影响。
3. 许多声称支持社会模型的参与者会赞同旨在通过教授适应技能来改变孤独谱系人士的支持措施，而非仅仅改革环境和社会，这与‘强’社会模型的原则不一致。

因此，这项混合方法研究与多个探究领域相关。它不可避免地涉及残疾研究与理论，而关于参与者及参与者子群体对特定干预目标感知可取性的发现也对实践具有意义。文章的部分内容涉及哲学、逻辑一致性与本体论。本研究是实证性的，也是一项科学假设检验的实践，但其发现也具有政治意义。

方法

参与者

本研究的样本亦在 Hersh等（2024）的研究中有所描述。参与者通过社交媒体以及联系多个国家的孤独谱系组织招募而来；为符合资格，他们必须是成年人与孤独谱系有所关联，例如身为孤独谱系人士、身为孤独谱系人士的家庭成员，或是孤独谱系专业人员/研究角色。在移除据推测无效/机器人/虚假数据以及无法评估其有效性的不完整回答后（参见补充附录A；另见Goodrich et al., 2023; Pellicano et al., 2023），保留了504份回答（278份来自孤独谱系人士，226份来自非孤独谱系人士）（参见表1）。大多数参与者来自加拿大（*n*=55）、英国（*n*=60）或美国（*n*=337）。

Table 1. Sample demographics.

	All autistic participants	Parents/Caregivers		Other family		Professionals		Researchers	
		Autistic	Non-autistic	Autistic	Non-autistic	Autistic	Non-Autistic	Autistic	Non-autistic
<i>n</i>	278	53	109	44	31	35	87	41	59
Age	36.15	44.63	47.04	38.24	37.37	43.00	39.01	37.07	33.04
Mean (SD)	(11.97)	(8.88)	(9.32)	(13.15)	(12.55)	(13.18)	(11.12)	(12.84)	(8.71)
RAADS-14 Mean (SD)	31.94	30.35	9.03	33.18	8.55	29.46	7.32	31.27	7.70
	(7.19)	(8.05)	(9.49)	(7.42)	(9.37)	(7.92)	(7.20)	(7.59)	(7.30)
Education ^a Mean (SD)	5.29	5.36	5.44	5.07	5.77	6.03	6.25	5.90	6.47
	(1.43)	(1.35)	(1.17)	(1.55)	(1.36)	(0.86)	(0.77)	(0.86)	(0.75)
Gender	165 F	40 F	97 F	25 F	24 F	21 F	73 F	24 F	45 F
	41 M	4 M	12 M	5 M	7 M	7 M	13 M	8 M	12 M
	72 NB	9 NB		14 NB		7 NB	1 NB	9 NB	2 NB
Non-Hispanic White (%)	84.56	76.92	83.33	82.57	77.42	82.35	72.41	80.00	71.19

F: female, M: male, NB: non-binary or other.

^aEducation is ranked from ‘Did not complete high school’ (1) to ‘Doctoral degree’ (7). See Supplemental Appendix.

Measures

NDM support/identification. Participants were asked whether they had heard of neurodiversity. If yes, NDM support/identification was measured on a 5-item, 7-point Likert-type scale adapted from the study by Nario-Redmond et al. (2013; see Supplemental Appendix B). Participants were also asked open-ended questions: ‘What do you think “neurodiversity” means?’ and ‘What do you think “the neurodiversity movement” means?’

Models of disability. If participants had heard of the SM or MM of disability (respectively), participants expressed support/opposition for them using single items (6-point Likert-type, Supplemental Appendix B). They were asked to define each model, and to comment on similarities and differences between each model and the NDM (open-ended, see Supplemental Appendix B).

Intervention views. Participants completed the Autism Intervention Attitudes Scale-General (AIAS-G) measuring support for different intervention goals using six Likert-type-scale response options (Baiden et al., 2024). The AIAS-G version administered in this study was adapted and edited to focus on a wide variety of autism intervention goals. In the same sample included in this study, Hersh et al. (2024) previously explored the dimensional structure of 35 AIAS-G items using exploratory graph analysis (Golino & Epskamp, 2017), finding five subscales: normalization (nine items), adaptive skill (eight items), well-being (six items), societal reform (five items) and supportive environment (five items) goals; two items were discarded. Item-level descriptive statistics and comparisons of autistic and non-autistic participants are presented in Supplementary Table C.3. In addition, participants answered single-item questions about who should choose intervention goals (autistic

people who can communicate, professionals, parents, autistic adult consultants) and whether spoken communication should be prioritized over other forms. Their views on curing, teaching useful skills to and making the world more supportive for people with anxiety, depression and epilepsy were solicited; they were also asked whether no intervention was needed for those conditions, and whether the appropriate approach varied between challenges (details in Supplemental Appendix B).

Qualitative content analysis

Data from open-ended questions were analysed using content analysis (Hsieh & Shannon, 2005; Kondracki et al., 2002). In general, our approach was inductive, though we endeavoured to make coding schemes comparable across questions where possible. However, related to our investigation of Hypothesis 3 regarding the ‘strong’ SM, some codes were formulated deductively, in order to explore the following:

- Who defines an SM of disability in a ‘strong’ way, by focusing only on the role of society in disabling people, excluding individual contributions to disability?
 - Which participants only support interventions and supports that aim at changing society and not interventions focusing on the individual?
- Which people believe the NDM entails an SM of disability?
 - And which people believe that it entails a ‘strong’ SM, one that excludes: individual contributions to disability and/or interventions focusing on the individual?

Questions regarding definitions of neurodiversity and the NDM, SM and MM were coded by P.D. and S.K.K. and those regarding similarities and differences between the

表1. 样本人口统计学特征

	所有孤独谱系参与者	父母/照护者		其他家庭成员		专业人员		研究人员	
		孤独谱系	非孤独谱系	孤独谱系	非孤独谱系	孤独谱系	非孤独谱系	孤独谱系	非孤独谱系
<i>n</i>	278	53	109	44	31	35	87	41	59
Age	36.15	44.63	47.04	38.24	37.37	43.00	39.01	37.07	33.04
均值（标准差）	(11.97)	(8.88)	(9.32)	(13.15)	(12.55)	(13.18)	(11.12)	(12.84)	(8.71)
RAADS-14 均值（标准差）	31.94	30.35	9.03	33.18	8.55	29.46	7.32	31.27	7.70
	(7.19)	(8.05)	(9.49)	(7.42)	(9.37)	(7.92)	(7.20)	(7.59)	(7.30)
教育程度 ^a 均值（标准差）	5.29	5.36	5.44	5.07	5.77	6.03	6.25	5.90	6.47
	(1.43)	(1.35)	(1.17)	(1.55)	(1.36)	(0.86)	(0.77)	(0.86)	(0.75)
性别	165 F	40 F	97 F	25 F	24 F	21 F	73 F	24 F	45 F
	41 M	4 M	12 M	5 M	7 M	7 M	13 M	8 M	12 M
	72 NB	9 NB		14 NB		7 NB	1 NB	9 NB	2 NB
非西班牙裔白人 (%)	84.56	76.92	83.33	82.57	77.42	82.35	72.41	80.00	71.19

F: 女性, M: 男性, NB: 非二元性别或其它。

^a教育程度从 ‘未完成高中’ （1）到 ‘博士学位’ （7）进行排序。参见补充附录。

措施

神经多样性模型支持/认同。参与者被问及是否听说过神经多样性。若答案为是，则神经多样性模型支持/认同通过一项改编自Nario-Redmond等（2013）研究的5条目、7点李克特式量表进行测量（参见补充附录B）。参与者还被问及开放式问题：‘你认为“神经多样性”是什么意思？’以及‘你认为“神经多样性运动”是什么意思？’

残疾模式。如果参与者听说过残疾的社会模型（SM）或医学模型（MM），他们将使用单项问题（6点李克特量表，补充附录B）表达对它们的支持或反对。参与者被要求定义每种模式，并评论每种模式与神经多样性模型（NDM）之间的相似性和差异（开放式问题，参见补充附录B）。

干预观点。参与者完成了自闭症干预态度量表-通用版（AIAS-G），该量表使用六点李克特量表式回答选项测量对不同干预目标的支持程度（Baiden等，2024）。本研究使用的AIAS-G版本经过改编和编辑，以关注广泛的孤独症干预目标。在本研究包含的同一样本中，Hersh等（2024）先前使用探索性图分析（Golino与Epskamp，2017）探索了35个AIAS-G项目的维度结构，发现了五个子量表：正常化（九个项目）、适应技能（八个项目）、幸福感（六个项目）、社会改革（五个项目）和支持性环境（五个项目）目标；两个项目被舍弃。项目层面的描述性统计以及孤独谱系与非孤独谱系参与者的比较见补充表C.3。

此外，参与者回答了关于谁应选择干预目标（能沟通的孤独谱系群体、专业人员、父母、孤独谱系成年顾问）的单项问题，以及口头交流是否应优先于其他形式。

我们征询了他们关于治愈、教授实用技能以及为患有焦虑、抑郁和癫痫的人创造一个更具支持性的世界的看法；同时也询问了是否无需对这些情况进行干预，以及针对不同挑战是否应采取不同的方法（详情见补充附录B）。

质性内容分析

开放式问题的数据采用内容分析（Hsieh与Shannon，2005; Kondracki等, 2002）进行分析。总体而言，我们的方法是归纳式的，但我们努力在可能的情况下使编码方案在不同问题间具有可比性。然而，与我们关于‘强’社会模型的假设3调查相关，部分编码是演绎式制定的，旨在探究以下内容：

- 谁以‘强’的方式定义残障的社会模型，即仅关注社会在致残中的作用，排除个体因素对残障的影响？
 - 哪些参与者仅支持旨在改变社会的干预和支持，而不支持聚焦个体的干预？
- 哪些人认为神经多样性模型包含残障的社会模型？
 - 哪些人认为它包含一种‘强’的社会模型，即排除：个体因素对残障的影响和/或聚焦个体的干预？

关于神经多样性、NDM、SM和MM定义的问题由P.D.和S.K.K.进行编码，而那些关于

NDM and the SMs and MMs were coded by A.N.G. and E.K. Each pair drafted coding schemes, which were circulated to all authors for edits, after which pair members separately coded 20% of the responses for each question; inter-rater agreement of 80% or higher was required for each code. Coders were blinded to participants’ autism status and other characteristics (unless these were disclosed in responses to the open-ended questions being coded). After reliability was attained, the remaining responses were coded by a single coder.

Quantitative analyses

Because of the non-normal and skewed distributions of many variables (e.g. see Figures 2 and 3, Supplementary Figures C.1–C.6), we used ordinal statistics.

Direct comparisons of participants from two groups (e.g. autistic and non-autistic participants) used Wilcoxon–Mann–Whitney tests with Cliff’s delta (Cliff, 1993; Torchiano, 2022) as an effect size. Comparisons of one group’s responses to ‘neutral’ responses (i.e. scores reflecting neither agreement nor disagreement) used one-sample Wilcoxon tests. False discovery corrections (Benjamini & Hochberg, 1995) were applied to each table of comparisons.

We used scaled, ranked regression to examine whether NDM support/identification scores (on the measure adapted from Nario-Redmond et al. (2013)) predicted SM, MM and AIAS intervention goal endorsement. This was analogous to using Spearman’s correlations, but allowed us to include autistic/non-autistic group (entered into analysis as sum-coded contrast factor) and its interaction with NDM support/identification, to control for group differences, and to determine whether group differences could be accounted for by NDM/support identification. We also used scaled, ranked regression when examining effects of multiple categorical variables simultaneously. False discovery corrections were applied to each table of comparisons.

Supplementary, exploratory comparisons of code endorsement frequencies across autistic and non-autistic participants are reported with both logistic regression and Fisher’s exact tests. No multiple comparison correction was applied to these exploratory analyses.

Community involvement

This study team was highly neurodiverse. All authors primarily responsible for coding qualitative data presented in this article (i.e. P.D., E.K., S.K.K. and A.N.G.) are autistic. Several other authors identify as non-autistic, albeit often with close family and other personal ties to autism. This project did not involve any participatory research methodology (e.g. community-based participatory research) with non-academic community members.

Results

Almost all participants had heard of neurodiversity (98.92% autistic, 95.58% non-autistic), and most had heard of the SM (79.86% autistic, 53.54% non-autistic) and MM (78.42% autistic, 65.02% non-autistic). Autistic people were more likely to have heard of each, $p \leq 0.03$ (see Supplementary Table C.1).

Autism identification in relation to views on neurodiversity, disability models and intervention

Hersh et al. (2024) previously examined whether being autistic versus non-autistic predicted NDM, SM, MM and intervention goal support in this dataset (see also Supplementary Table C.2; single items in Supplementary Table C.3).⁴ Here, we further examine this question in sub-groups: among parents/caregivers, other family, professionals and researchers (see Supplementary Tables C.4–C.7), finding results generally similar to those in the full sample (see Supplementary Table C.2). However, differences in viewpoints based on autism identification were most apparent among parents/caregivers. For example, autistic parents/caregivers endorsed the SM more than non-autistic parents/caregivers, but differences in SM support between autistic and non-autistic participants were not apparent among other family members, professionals, or researchers. Similarly, only autistic parents/caregivers rated well-being goals significantly lower than non-autistic parents/caregivers (likely reflecting concerns about how those goals would be pursued⁵); differential ratings of well-being goals for autistic participants were not observed for other subgroups.

Means and group comparisons from additional single-item questions are presented in Supplementary Tables C.8–C.12. Interestingly, autistic participants – including autistic parents/caregivers and professionals, respectively – much less often endorsed parents and trained professionals having leading roles in setting intervention goals, and more often supported involvement of autistic consultants in developing goals for young children. Autistic participants also less often supported promoting communication primarily through verbal speech.

Views on intervention objectives for co-occurring conditions (anxiety, depression and epilepsy) were overall similar across groups, but autistic participants sometimes favoured efforts to make a supportive world more than non-autistics, and were sometimes more sceptical of cure- or skill-building-oriented interventions (see Supplementary Tables C.13–C.17).

Correlates of NDM support

To examine our first hypothesis – that neurodiversity support would be associated with decreased endorsement of

NDM与SMs和MMs之间相似性与差异性的问题则由 A.N.G.和E.K.编码。每对编码者起草了编码方案，并分发给所有作者进行修改，之后每对成员分别对每个问题的20%回答进行编码；每个编码类别要求达到80%或更高的评分者间一致性。编码者对参与者的孤独谱系状况及其他特征（除非这些信息在所编码的开放式问题回答中被披露）保持盲态。在可靠性达标后，其余回答由单一编码者完成编码。

定量分析

由于许多变量的分布非正态且存在偏态（例如参见图2和图3，补充图C.1–C.6），我们采用了序数统计方法。

两组参与者（例如孤独谱系与非孤独谱系参与者）的直接比较采用Wilcoxon–Mann-Whitney检验，并以Cliff’s delta（Cliff, 1993; Torchiano, 2022）作为效应量。单组反应与'中性'反应（即既不表示同意也不表示反对的分数）的比较采用单样本Wilcoxon检验。每个比较表均应用了错误发现率校正（Benjamini与Hochberg, 1995）。

我们使用标准化秩次回归来检验神经多样性模型支持/认同分数（基于Nario-Redmond等（2013）改编的测量工具）是否预测了社会模型、医学模型和AIAS干预目标的认同度。这类似于使用Spearman相关性分析，但允许我们将孤独谱系/非孤独谱系组别（以求和编码的对比因子形式纳入分析）及其与神经多样性模型支持/认同的交互作用纳入模型，以控制组间差异，并确定组间差异是否可由神经多样性模型/支持认同来解释。在同时考察多个分类变量的效应时，我们也使用了标准化秩次回归。每个比较表均应用了错误发现率校正。

关于孤独谱系与非孤独谱系参与者之间编码认同频率的补充性探索性比较，已通过逻辑回归和Fisher精确检验进行报告。这些探索性分析未应用多重比较校正。

社区参与

本研究团队具有高度的神经多元性。所有主要负责本文定性数据编码的作者（即P.D.、E.K.、S.K.K.和A.N.G.）均为孤独谱系人士。其他几位作者则认同为非孤独谱系，但通常与孤独谱系有密切的家庭成员或其他个人联系。本项目未采用任何与非学术社区成员合作的参与式研究方法（例如社区参与式研究）。

结果

几乎所有参与者都听说过神经多样性（孤独谱系群体98.92%，非孤独谱系95.58%），大多数人听说过社会模型（孤独谱系群体79.86%，非孤独谱系53.54%）和医学模型（孤独谱系群体78.42%，非孤独谱系65.02%）。孤独谱系群体对每一项的知晓率都更高， $p \leq 0.03$ （参见补充表C.1）。

孤独谱系认同与对神经多样性、残障模型及干预的看法之间的关系

Hersh等（2024）先前已在此数据集（另见补充表C.2；补充表C.3中的单项条目）中检验了孤独谱系与非孤独谱系身份是否能预测对NDM、SM、MM及干预目标的支持。⁴ 在此，我们进一步在子群体中探讨此问题：在父母/照护者、其他家庭成员、专业人员及研究人员中（见补充表C.4–C.7），发现结果总体上与全样本中的结果相似（见补充表C.2）。然而，基于孤独谱系认同的观点差异在父母/照护者中最为明显。例如，孤独谱系父母/照护者对SM的认同高于非孤独谱系父母/照护者，但孤独谱系与非孤独谱系参与者之间对SM支持的差异在其他家庭成员、专业人员或研究人员中并不明显。同样，只有孤独谱系父母/照护者对福祉目标的评分显著低于非孤独谱系父母/照护者（这可能反映了对如何追求这些目标的担忧⁵）；在其他子群体中，并未观察到孤独谱系参与者对福祉目标的评分差异。

补充单项问题的均值与组间比较结果见附表C.8–C.12。有趣的是，孤独谱系参与者——包括孤独谱系父母/照护者和专业人员——较少认可父母和受过培训的专业人员在设定干预目标中发挥主导作用，而更常支持让孤独谱系顾问参与制定幼儿的干预目标。孤独谱系参与者也更少支持主要通过口语来促进沟通。

对于共患条件（焦虑、抑郁和癫痫）的干预目标，各组的看法总体相似，但孤独谱系参与者有时比非孤独谱系群体更倾向于努力打造一个支持性环境，有时也对治愈导向或技能培养导向干预持更怀疑态度（见附表C.13–C.17）。

NDM支持度的相关因素

为了检验我们的第一个假设——即神经多样性支持将与

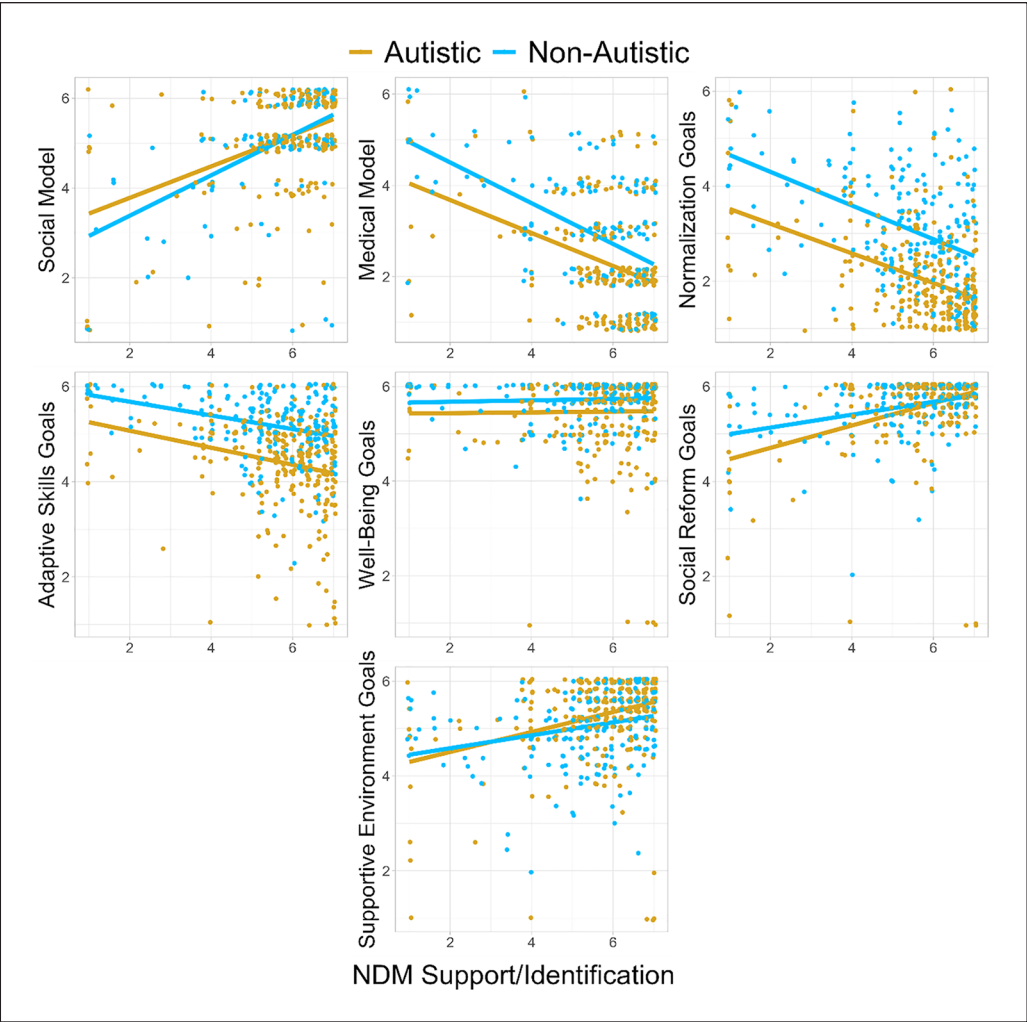


Figure 2. Linear slopes representing associations between NDM support/identification and support for the SM, the MM and AIAS intervention goal types, separately for autistic and non-autistic participants. Data points are slightly jittered to reduce overlap. Note that these linear slopes differ from the ordinal statistical analysis, and are displayed only for visualization, so that readers can view the original raw data points prior to their conversion to ranks. While there was no linear association between NDM support/identification and endorsement of well-being intervention goals, they were ordinally associated (Supplementary Table C.18, Supplementary Figure C.2).

normalization/behaviour reduction intervention goals, but not with altered endorsement of adaptive skill and well-being goals – we used scaled, ranked regression to investigate associations between NDM support/identification and AIAS subscale scores.

NDM support/identification predicted greater support for the SM and for societal reform and supportive environment intervention goals, corrected $p < 0.0001$ (see Figure 2; Supplementary Table C.18). It also ordinally predicted greater support for well-being intervention goals, $p = 0.004$. NDM support/identification predicted reduced endorsement of the MM and normalization and adaptive skills intervention goals, $p < 0.0001$.

In relation to the second hypothesis (that autistic neurodiversity supporters would be less susceptible to ‘neurodiversity lite’ rhetoric and therefore would be more

enthusiastic about societal reform than non-autistic neurodiversity supporters), autistic and non-autistic people did not statistically differ in endorsement of societal reform and supportive environments when accounting for NDM support (see Supplementary Table C.18). However, autistic participants continued to endorse the MM and normalization, adaptive skill, and well-being intervention goals less than non-autistics in models that included NDM support, $p \leq 0.0001$ (see Supplementary Table C.18): thus, autistic people tended to like these goals less even when compared to non-autistics who considered themselves equally NDM-supportive. Autism status and NDM support/identification did not interact significantly, uncorrected $p \geq 0.13$ (see Supplementary Table C.18).

NDM support/identification was related to greater endorsement of autistic people playing a leading role in

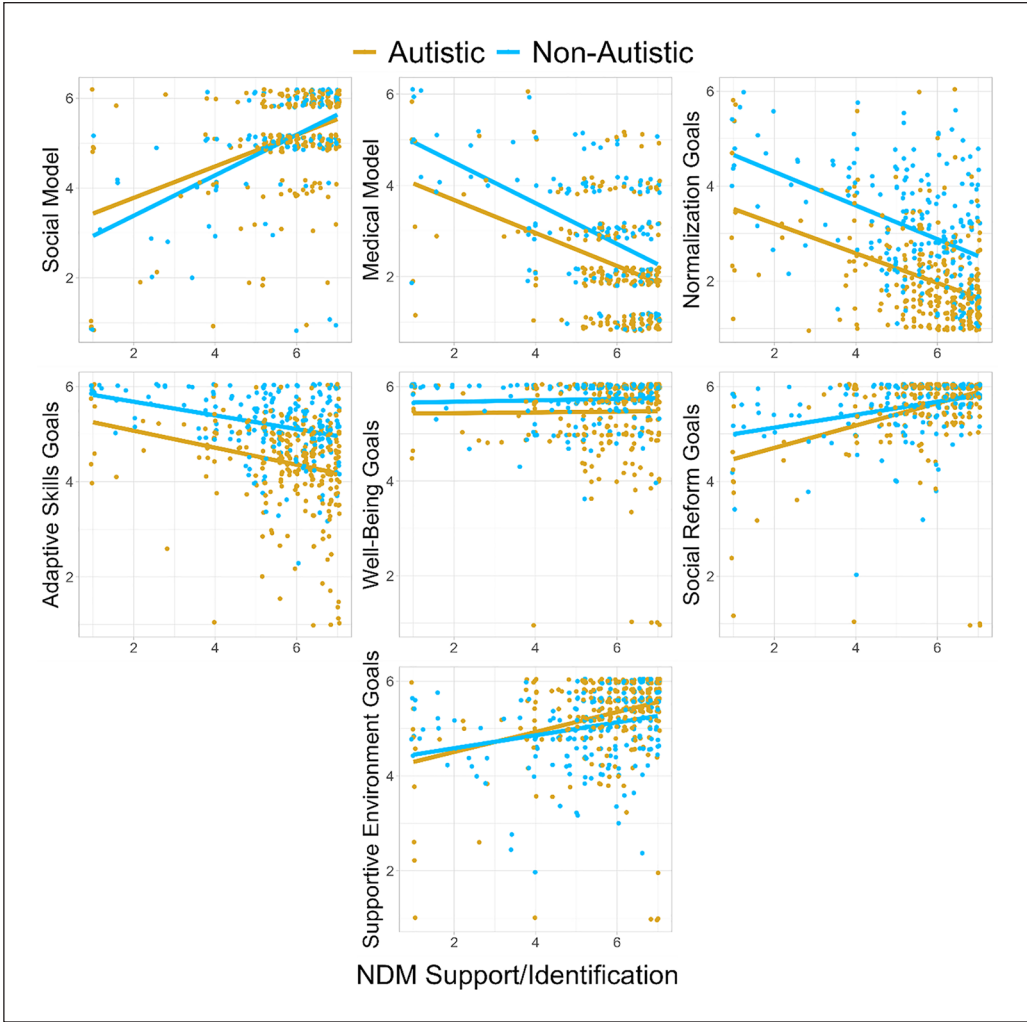


图 2. 线性斜率代表神经多样性模型支持/认同与对社会模型、医学模型和AIAS干预目标类型支持之间的关联，分别针对孤独谱系和非孤独谱系参与者。数据点轻微抖动以减少重叠。请注意，这些线性斜率与顺序统计分析不同，仅为可视化展示，以便读者能在数据转换为秩次前查看原始数据点。虽然神经多样性模型支持/认同与幸福感干预目标的认同之间没有线性关联，但它们存在顺序关联（补充表 C.18，补充图 C.2）。

正常化/行为减少干预目标的认同度降低相关，但与适应技能和福祉目标的认同度变化无关——我们使用了标准化秩次回归来研究神经多样性模型支持/认同与AIAS 分量表得分之间的关联。

神经多样性模型支持/认同预测了对社会模型以及社会改革和支持性环境干预目标更大的支持，已校正 $p < 0.0001$ (见图2；补充表 C.18)。它还顺序预测了对幸福感干预目标更大的支持， $p = 0.004$ 。神经多样性模型支持/认同预测了对医学模型以及正常化和适应技能干预目标的认同降低， $p < 0.0001$ 。

关于第二个假设（即孤独谱系神经多样性支持者可能更不易受‘简化版神经多样性’言论影响，因此会比非孤独谱系神经多样性支持者更

热衷于社会改革），在考虑NDM支持的情况下，孤独谱系群体与非孤独谱系群体在社会改革和支持性环境的认同程度上并无统计学差异（参见补充表 C.18）。然而，在包含NDM支持的模型中，孤独谱系参与者对医学模型、正常化、适应技能以及幸福感干预目标的认同程度仍低于非孤独谱系群体， $p \leq 0.0001$ （参见补充表 C.18）：因此，即使与同样认为自己是NDM支持者的非孤独谱系群体相比，孤独谱系群体对这些目标的喜好程度也倾向于更低。孤独谱系状态与NDM支持/认同之间没有显著的交互作用，未经校正的 $p \geq 0.13$ （参见补充表 C.18）。

神经多样性模型支持/认同与更倾向于支持孤独谱系群体在

setting intervention goals, and involving autistic consultants when interventions are for young children, as well as of promoting communication using any technique. NDM support/identification was associated with reduced support for parents and professionals playing leading roles and for focusing primarily on verbal speech (see Supplementary Table C.19, Supplementary Figure C.3). Neurodiversity support/identification predicted robustly greater enthusiasm for making the world more supportive for people with anxiety, depression and epilepsy, and modestly reduced support for curing those conditions (Supplementary Table C.20, Supplementary Figures C.4–C.6). Interestingly, NDM supporters – at least among autistic participants – were slightly less likely to state people with anxiety did not face challenges or need support, although few participants expressed that opinion.

Open-ended responses

Open-ended responses are summarized in greater detail in Supplemental Appendix E, which presents coding schemes, frequencies of code endorsement and illustrative quotations (see Supplementary Tables E.1–E.8). A subset of codes is briefly narratively summarized below.

Definitions of neurodiversity. ‘Neurodiversity’ was typically understood as a descriptive term (81.06%), referring to neurocognitive diversity in everyone (54.38%) or neurodivergent people (25.46%; see Supplementary Table E.1). However, some definitions implied neurodiversity is an inherently value-laden and prescriptive term (29.33%) and, in that vein, many responses suggested it values diversity (30.75%). Most respondents referred to biology/neurobiology (63.34%) and many alluded to cognition/perception (31.57%); behaviour was seldom mentioned (9.57%).

Definitions of the NDM. The NDM was almost universally understood as prescriptive (96.10%), though exploratory comparisons suggested autistics (66.18%) were more likely than non-autistics (55.81%) to suggest it advocates for neurodivergent people (vs autistic people or all humans), uncorrected $p=0.02$ (see Supplementary Table E.2). More non-autistic (65.12%) than autistic (51.84%) participants talked about the NDM accepting/valuing differences, $p=0.003$, whereas more autistic (60.66%) than non-autistic (48.84%) participants, $p=0.009$, discussed the NDM changing society – notably, through seeking rights (15.81% autistic, 6.51% non-autistic, $p=0.002$) and opposing stigma/discrimination (11.76% autistic, 4.65% non-autistic, $p=0.007$). Furthermore, autistic people (22.79%) more often than non-autistics (14.42%), $p=0.02$, suggested that the NDM promotes the thriving of neurodivergent people. Across neurotypes, around a quarter of participants suggested the NDM opposes pathologization (27.10%).

Definitions of the SM. Participants overwhelmingly agreed the SM postulates society is involved in causing disability (84.71%; see Supplementary Table E.3). Most participants provided ‘strong’ definitions of the SM, indicating society constructs disability (60.29%), but some suggested society causes most issues (11.47%) or exacerbates issues (6.47%), or that person–environment misfit causes disability (4.41%). Participants frequently mentioned the SM’s focus on structural/systemic issues, such as barriers, inaccessibility, lack of supports and social norms (68.53%), and some respondents discussed oppression faced by disabled people (19.71%). Many suggested the SM rejects medicalization/pathologization (34.41%).

Definitions of the MM. Most participants indicated the MM postulates that individuals’ own characteristics contribute to disability (58.68%); indeed, ‘weak’ forms of the MM, with societal factors also contributing, were rarely described (2.48%; see Supplementary Table E.4). Disability in the MM was often described as rooted in biology (24.52%). Participants described the MM as a pathology-based viewpoint (68.60%), suggesting it argues intervention should change individuals (49.04%), including through cure/normalization (43.53%).

Similarities of NDM and SM. Many participants suggested the NDM and SM share common goals and attitudes (52.44%), such as promoting accessibility (27.44%) and inclusion and acceptance (38.11%; see Supplementary Table E.5). Some responses indicated that NDM and SM share a ‘strong’ view that disability is socially constructed (19.21%). The models were described as agreeing that disability is neutral (12.80%)/not negative (11.59%). Some responses suggested that NDM emerged from the SM (15.55%).

Differences between the NDM and SM. Some participants simply denied the existence of differences between the NDM and SM (18.94%; see Supplementary Table E.6). Others suggested they have different goals and attitudes (15.28%): for example, the SM advocates more for accommodations (3.67%), or the NDM advocates more for acceptance/inclusion (6.98%). The SM was noted to apply to more disabilities/diagnoses (13.00%). Certain responses suggested that NDM is practical/applied and the SM philosophical/theoretical (8.97%). Notably, some participants suggested the models have different views on the origin of disability (19.27%), with the NDM recognizing medical needs that cannot be addressed through societal reform (12.96%) or accepting both individual and societal contributions to disability (9.30%), in contrast to an SM in which disability is socially constructed (8.97%).

Similarities between the NDM and MM. Numerous responses asserted the NDM and MM are not similar

设定干预目标、在为幼儿设计干预时让孤独谱系顾问参与，以及推广使用任何技术进行沟通方面发挥主导作用相关。神经多样性模型支持/认同与减少对父母和专业人员发挥主导作用以及主要关注口语的支持相关（参见补充表C.19，补充图C.3）。神经多样性支持/认同有力地预测了对让世界对患有焦虑、抑郁和癫痫的人群更具支持性抱有更大的热情，并适度降低了对治愈这些状况的支持（补充表C.20，补充图C.4–C.6）。有趣的是，神经多样性模型支持者——至少在孤独谱系参与者中——略微不太可能声称患有焦虑的人不面临挑战或不需要支持，尽管表达此观点的参与者很少。

开放式回答

开放式回答在补充附录E中有更详细的总结，其中呈现了编码方案、编码采纳频率及示例引述（参见补充表E.1–E.8）。下文简要叙述性地总结了一部分编码。

神经多样性的定义。“神经多样性”通常被理解为一个描述性术语（81.06%），指所有人（54.38%）或神经多样性人群（25.46%；参见补充表E.1）的神经认知多样性。然而，一些定义暗示神经多样性是一个本质上带有价值判断和规范性的术语（29.33%），并且在此脉络下，许多回答暗示它重视多样性（30.75%）。大多数受访者提及生物学/神经生物学（63.34%），许多人提及认知/感知（31.57%）；行为很少被提及（9.57%）。

NDM的定义。NDM几乎被普遍理解为规范性模型（96.10%），但探索性比较表明，孤独谱系群体（66.18%）比非孤独谱系群体（55.81%）更倾向于认为它倡导神经殊异群体（而非孤独谱系群体或全体人类），未校正 $p=0.02$ （参见补充表E.2）。更多非孤独谱系（65.12%）而非孤独谱系（51.84%）参与者谈到NDM接纳/重视差异， $p=0.003$ ；而更多孤独谱系（60.66%）而非非孤独谱系（48.84%）参与者， $p=0.009$ ，讨论了NDM改变社会——特别是通过争取权利（15.81%孤独谱系，6.51%非孤独谱系， $p=0.002$ ）和反对污名/歧视（11.76%孤独谱系，4.65%非孤独谱系， $p=0.007$ ）。此外，孤独谱系群体（22.79%）比非孤独谱系群体（14.42%）更常提出NDM促进神经殊异群体的蓬勃发展， $p=0.02$ 。在不同神经类型中，约四分之一的参与者认为NDM反对病理化（27.10%）。

SM的定义。参与者绝大多数同意SM假设社会参与导致残障（84.71%；参见补充表E.3）。大多数参与者提供了SM的“强”定义，表明社会建构了残障（60.29%），但有些人认为社会导致了大部分问题（11.47%）或加剧了问题（6.47%），或认为人与环境不匹配导致了残障（4.41%）。参与者频繁提到SM关注结构性/系统性问题，如障碍、不可访问性、缺乏支持和社会规范（68.53%），一些受访者讨论了残障群体面临的压迫（19.71%）。许多人认为SM拒绝医疗化/病理化（34.41%）。

医学模型的定义。大多数参与者指出医学模型假设个体自身的特征会导致残障（58.68%）；事实上，描述“弱”形式的医学模型（社会因素也起作用）的情况很少（2.48%；参见补充表E.4）。医学模型中的残障常被描述为根植于生物学（24.52%）。参与者将医学模型描述为一种基于病理学的观点（68.60%），暗示它主张干预应改变个体（49.04%），包括通过治愈/正常化（43.53%）。

NDM与SM的相似之处。许多参与者认为NDM与SM拥有共同的目标和态度（52.44%），例如促进无障碍环境（27.44%）以及包容与接纳（38.11%；参见补充表E.5）。部分回答指出NDM与SM都持有一种“强烈”的观点，即残障是社会建构的（19.21%）。这些模型被描述为一致认为残障是中性的（12.80%）/并非负面的（11.59%）。一些回答认为NDM源自SM（15.55%）。

NDM与SM之间的差异。部分参与者直接否认NDM与SM之间存在差异（18.94%；参见补充表E.6）。另一些人则认为它们有不同的目标和态度（15.28%）：例如，SM更倡导提供便利措施（3.67%），而NDM更倡导接纳/包容（6.98%）。SM被认为适用于更多残障/诊断类别（13.00%）。某些回答提出NDM是实用/应用型的，而SM是哲学/理论型的（8.97%）。值得注意的是，一些参与者认为这两种模型对残障的起源持有不同观点（19.27%），NDM承认存在无法通过社会改革解决的医疗需求（12.96%）或同时接受个体与社会因素对残障的贡献（9.30%），这与SM中残障是社会建构的观点（8.97%）形成对比。

NDM与MM之间的相似性。许多回应断言NDM和MM并不相似

(46.89%; see Supplementary Table E.7). However, some suggested they have common goals and attitudes (33.62%): for example, recognizing the reality of differences, disability and challenges (14.41%), advocating for supports/intervention (9.04%), ascribing biological origins to disability (7.91%), and acknowledging medical needs (4.24%). A few responses commented on their shared propensity for dividing the world into typical/divergent binaries (4.24%).

Differences between the NDM and MM. Most respondents felt that goals and attitudes of the NDM and MM were dissimilar (56.82%) in terms of their acceptance of diversity (12.50%), endorsement of societal reform (23.01%) and desire to change individuals (38.07%; see Supplementary Table E.8). Regarding the latter, responses suggested cure/prevention is promoted by the MM (20.17%) and opposed by the NDM (10.80%), and that treatment/intervention is promoted by the MM (22.44%) and opposed by the NDM (5.11%). Participants also commented on the models’ different views of deficits/strengths differences (43.75%), with many indicating the MM pathologizes differences (28.98%) but that the NDM does not (30.68%). Some described disability arising in the individual in the MM (7.10%) but being socially constructed in the NDM (7.39%). Certain respondents simply suggested that NDM and MM are completely different (13.07%).

Qualitative–quantitative triangulation

To examine our third hypothesis –that many participants claiming to support the ‘strong’ SM would express views that seem inconsistent with it, potentially confusing debates regarding neurodiversity and making it more difficult to resolve disagreements – we defined ‘strong’ SM supporters as the subset of participants (108 autistic, 51 non-autistic) who quantitatively supported the SM and described it in ‘strong’ terms, with societal and not individual characteristics constructing disability (see Supplemental Appendix D for details). One-sample Wilcoxon tests examined whether their support for AIAS intervention goals differed from neutrality (scores of 3.5: between ‘Slightly Disagree’ and ‘Slightly Agree’); this was also done for single items regarding conventional, speech-oriented versus atypical/augmentative/alternative approaches to promoting communication and single items regarding person- versus societal-oriented interventions for anxiety, depression and epilepsy (see Figure 3).

Overall, ‘strong’ SM-supporting participants had more-negative-than-neutral views of AIAS normalization intervention goals and of focusing primarily on verbal speech over other forms of communication, $p < 0.0001$ (see Figure 3, Supplementary Tables D.1, D.4). However, they had more-positive-than-neutral

opinions towards AIAS adaptive skills intervention goals; curing depression and epilepsy; and teaching useful skills to people with anxiety, depression and epilepsy, $p \leq 0.002$, even though these involve changing the individual, seemingly contrary to the ‘strong’ SM’s sole focus on society. ‘Strong’ SM-supporting participants also enthusiastically supported AIAS well-being, societal reform, supportive environment goals and changing the world to be more supportive of people with anxiety, depression and epilepsy, $p < 0.0001$. Similar results held among autistic and non-autistic participants separately (see Supplementary Tables D.2, D.3, D.5 and D.6), although non-autistic ‘strong’ SM supporters’ views on curing depression did not significantly depart from neutrality. Identical patterns of statistical significance were obtained when analyses incorporated information from responses to ND/NDM questions to better classify participants as having ‘strong’ (84 autistic, 39 non-autistic) views or not (see Supplementary Tables D.7–D.12).

We also used scaled, ranked regression (controlling for differences between autistic and non-autistic participants) to compare ‘strong’ SM supporters to participants who supported a ‘weak’ SM (44 autistic, 27 non-autistic). We found no statistical differences between ‘strong’ and ‘weak’ supporters in their views on AIAS intervention goals, spoken versus any communication items and person- versus societal-focused intervention items for epilepsy, anxiety and depression, corrected $p \geq 0.17$ (see Supplementary Tables D.1, D.6), though before correction there was a tendency for ‘strong’ supporters to be more enthusiastic about supportive environment goals, uncorrected $p = 0.03$. Similar results were obtained when ND/NDM questions were used to better classify participants as having ‘strong’ or ‘weak’ views (see Supplementary Tables D.7–D.12).

Discussion

This study used mixed methods to richly characterize understandings of neurodiversity and models of disability in a large sample of autism community members. As previously reported by Hersh et al. (2024), participants generally supported the NDM and SM of disability, while opposing the MM. Participants were also highly aware of neurodiversity, and high awareness of neurodiversity has previously been linked to support for NDM-aligned views (Kapp et al., 2013).

Social construction of disability

One of the key topics investigated here is autism community members’ views on the social construction of disability. Many critiques of the NDM connect it to the SM, arguing that the ‘strong’ SM’s denial of individual

(46.89%; 参见补充表 E.7)。然而，一些人认为它们有共同的目标和态度 (33.62%): 例如，承认差异、残疾和挑战的现实 (14.41%), 倡导支持/干预 (9.04%), 将残疾归因于生物起源 (7.91%), 以及承认医疗需求 (4.24%)。少数回应评论了它们共同的倾向，即将世界划分为典型/差异二元对立 (4.24%)。

NDM 与 MM 之间的差异。大多数受访者认为 NDM 与 MM 的目标和态度在接纳多样性 (12.50%)、支持社会改革 (23.01%) 以及希望改变个体 (38.07%; 见补充表 E.8) 方面存在差异 (56.82%)。关于后者，回应表明 MM 提倡治愈/预防 (20.17%) 而 NDM 反对 (10.80%), 并且 MM 提倡治疗/干预 (22.44%) 而 NDM 反对 (5.11%)。参与者还评论了两种模型对缺陷/优势差异的不同看法 (43.75%), 许多人指出 MM 将差异病理化 (28.98%) 而 NDM 则不这样做 (30.68%)。一些人描述 MM 中残障源于个体 (7.10%) 而在 NDM 中则是社会建构的 (7.39%)。某些受访者直接表示 NDM 与 MM 完全不同 (13.07%)。

定性-定量三角验证

为了检验我们的第三个假设——即许多声称支持 ‘强’ 社会模型的参与者会表达出看似与之不一致的观点，这可能混淆关于神经多样性的辩论，并使解决分歧变得更加困难——我们将 ‘强’ 社会模型支持者定义为那些在定量上支持社会模型并用 ‘强’ 术语描述它 (认为残障是由社会而非个体特征构建) 的参与者集 (108名孤独谱系参与者, 51名非孤独谱系参与者) (详见补充附录 D)。单样本 Wilcoxon 检验考察了他们对于 AIAS 干预目标的支持是否偏离中立 (评分 3.5: 介于 ‘略微不同意’ 和 ‘略微同意’ 之间); 同样也对关于促进沟通的传统、言语导向方法与非典型/增强/替代方法的单项条目, 以及关于焦虑、抑郁和癫痫的个人导向与社会导向干预的单项条目进行了检验 (见图 3)。

总体而言, 支持 ‘强’ SM 的参与者对 AIAS 正常化干预目标以及优先关注口头言语而非其他沟通形式持有比中立更负面的看法, $p < 0.0001$ (参见图 3、补充表 D.1、D.4)。然而, 他们对

AIAS 适应技能干预目标; 治愈抑郁和癫痫; 以及向患有焦虑、抑郁和癫痫的人群教授实用技能持比中立更积极的看法, $p \leq 0.002$, 尽管这些涉及改变个体, 似乎与 ‘强’ SM 仅关注社会的立场相悖。支持 ‘强’ SM 的参与者还热情支持 AIAS 幸福感、社会改革、支持性环境目标以及改变世界使其更支持患有焦虑、抑郁和癫痫的人群, $p < 0.0001$ 。类似结果在孤独谱系和非孤独谱系参与者中分别成立 (参见补充表 D.2、D.3、D.5 和 D.6), 尽管非孤独谱系的 ‘强’ SM 支持者对治愈抑郁的看法并未显著偏离中立。当分析纳入对 ND/NDM 问题的回答信息以更好地将参与者分类为持有 ‘强’ 观点 (84 名孤独谱系, 39 名非孤独谱系) 或不持有时 (参见补充表 D.7–D.12), 获得了完全相同的统计显著性模式。

我们还使用了标准化秩次回归 (控制孤独谱系与非孤独谱系参与者之间的差异) 来比较 ‘强’ 社会模型支持者与支持 ‘弱’ 社会模型的参与者 (44 名孤独谱系, 27 名非孤独谱系)。我们发现 ‘强’ 与 ‘弱’ 支持者在他们对 AIAS 干预目标、言语沟通与任何沟通条目以及癫痫、焦虑和抑郁的个人导向与社会导向干预条目的看法上没有统计学差异, 校正后 $p \geq 0.17$ (见补充表 D.1, D.6), 不过在校正前, ‘强’ 支持者倾向于对支持性环境目标更热情, 未校正 $p = 0.03$ 。当使用神经多样性/神经多样性模型问题来更好地将参与者分类为持有 ‘强’ 或 ‘弱’ 观点时, 也得到了类似的结果 (见补充表 D.7–D.12)。

讨论

本研究采用混合方法, 在一个大规模的孤独谱系社群成员样本中, 深入刻画了对神经多样性及残疾模式的认知。正如 Hersh 等 (2024) 先前报告的那样, 参与者普遍支持神经多样性模型 (NDM) 和社会模型 (SM) 的残疾观, 同时反对医学模型 (MM)。参与者对神经多样性也有高度认知, 而高水平的神经多样性认知此前已被证实与支持 NDM 相关观点相关联 (Kapp et al., 2013)。

残疾的社会建构

此处探讨的一个关键主题是孤独谱系群体成员对残疾社会建构的看法。许多对 NDM 的批评将其与 SM 联系起来, 认为 ‘强’ SM 否认个体

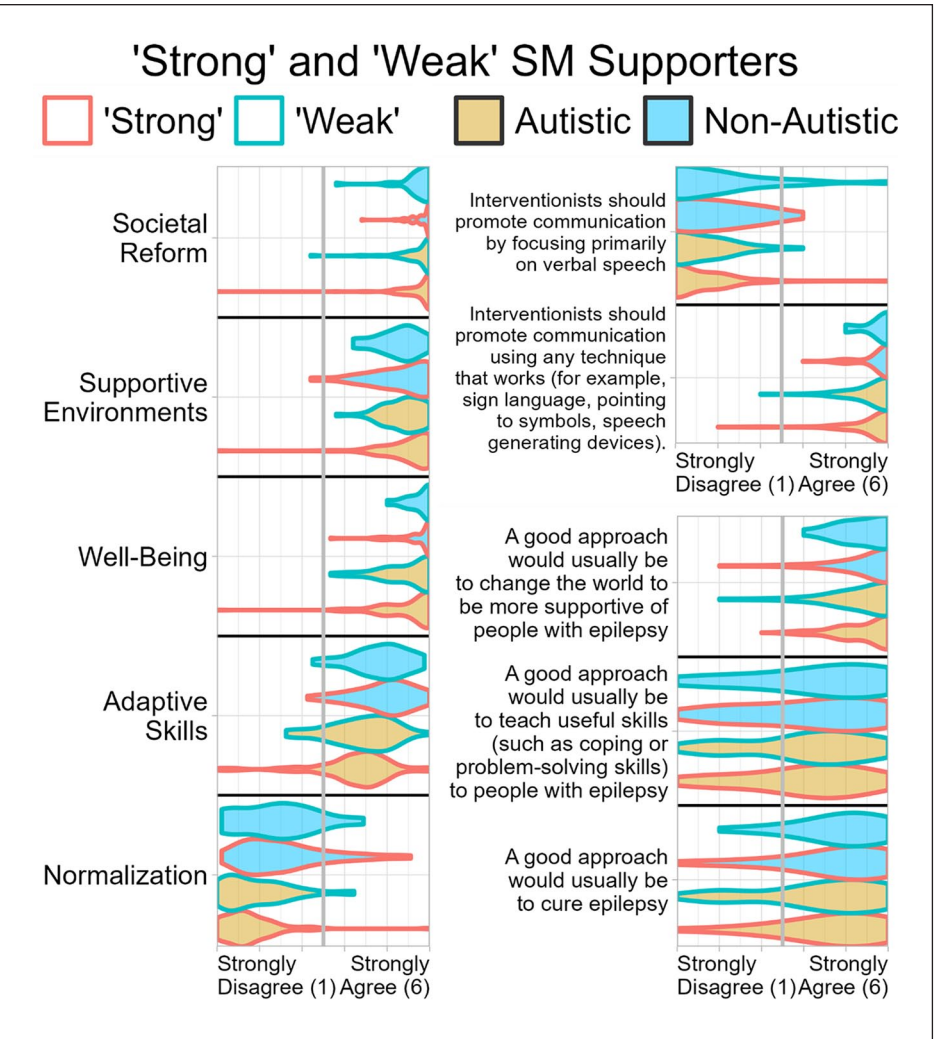


Figure 3. Violin plots depicting ratings of intervention goal types from the AIAS (left), approaches towards promoting communication (top right) and, as an example, co-occurring condition of interventions for people with epilepsy (bottom right), as a function of whether participants identified as autistic and whether their open-ended descriptions of the SM followed ‘strong’ understandings, in which society alone constructs disability (n = 159) or ‘weak’ understandings, with interacting societal and individual contributions creating disability (n = 71). A vertical grey line at 3.5 (midway between ‘Slightly Disagree’ and ‘Slightly Agree’) represents neutrality. Analyses also examined views on anxiety and depression intervention (see Supplemental Appendix D).

characteristics’ disabling effects is inappropriate for some or all autistic people (J. A. Hughes, 2021; Nelson, 2021). Some of this study’s participants were not familiar with the SM, but among those who were, the SM was generally seen as similar to the NDM. Moreover, most supported the SM and provided ‘strong’ definitions of it, noting society’s construction of disability without recognizing an influence of individual characteristics. While a considerable minority put forward ‘weak’ definitions, remaining open to the possibility individuals’ characteristics might have disabling effects independent of society, one might be tempted to conclude from solely quantitative data that NDM critics’ concerns are justified.

However, when open-ended responses about neurodiversity/the NDM were considered (not just those about the SM itself), the number of responses expressing ‘strong’ and ‘weak’ views became roughly even. Even more crucially, as per our third hypothesis, ‘strong’ SM supporters were generally comfortable with interventions aiming to change people by teaching adaptive skills. Furthermore, most ‘strong’ SM supporters endorsed curing depression and epilepsy. At first glance, it might seem contradictory for people who deny individual characteristics a role in disability to then support interventions to change these characteristics. Several interpretations seem possible.

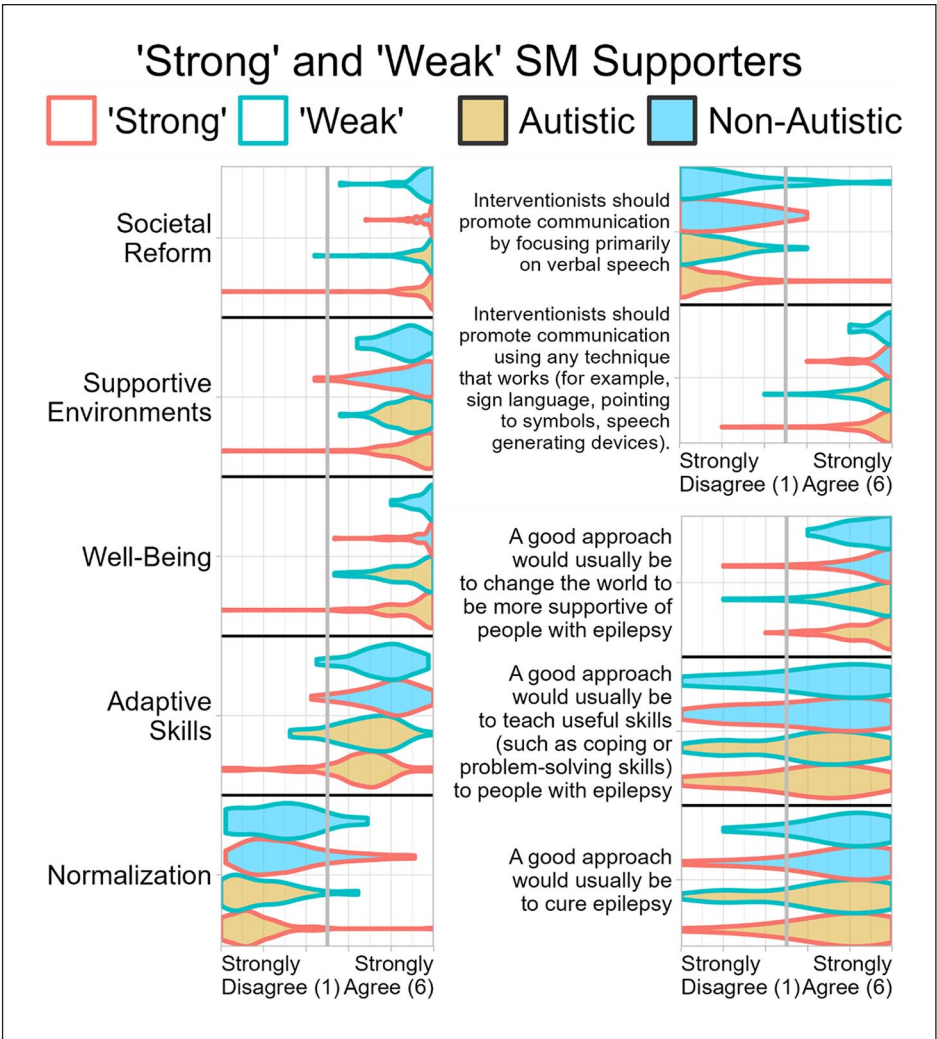


图 3. 小提琴图描绘了AIAS（左）对于干预目标类型的评分、促进沟通的方法（右上）以及，作为一个示例，针对癫痫患者的干预中并存状况（右下）的评分，这些评分作为参与者是否自我认同为孤独谱系以及他们对社会模型的开放式描述是否遵循“强”理解（即仅由社会建构残障，n = 159）或“弱”理解（即社会和个体因素相互作用共同创造残障，n = 71）的函数。一条位于3.5（即“轻微不同意”和“轻微同意”之间）的垂直灰色线代表中立。分析还考察了关于焦虑和抑郁干预的观点（参见补充附录D）。

特征’的致残效应对部分或所有孤独谱系群体是不恰当的 (J. A. Hughes, 2021; Nelson, 2021)。本研究的部分参与者并不熟悉社会模型，但在熟悉者中，社会模型通常被视为与神经多样性模型相似。此外，大多数人支持社会模型，并给出了对其的“强”定义，指出社会对残障的建构，而未认识到个体因素的影响。尽管相当一部分少数派提出了“弱”定义，仍对个体特征可能独立于社会而产生致残效应的可能性持开放态度，但从定量数据来看，人们可能会倾向于得出神经多样性模型批评者的担忧是合理的结论。

然而，当考虑到关于神经多样性/NDM的开放式回答（不仅仅是关于SM本身的回答）时，表达‘强’和‘弱’观点的回答数量大致持平。更重要的是，根据我们的第三个假设，‘强’SM支持者通常对旨在通过教授适应技能来改变人们的干预感到满意。此外，大多数‘强’SM支持者赞同治愈抑郁和癫痫。乍一看，否认个体特征在残障中起作用的人，转而支持改变这些特征的干预，似乎有些矛盾。几种解释似乎是可能的。

First, although ultimately viewing society as responsible, and desiring a societal solution, ‘strong’ SM advocates might be open to individual-focused interventions, at least in some contexts, due to the difficulty of reforming society.

Second, participants might support ‘strong’ SM views only in relation to particular disabilities or characteristics (Ne’eman & Pellicano, 2022), with adaptive skills, depression and epilepsy falling (wholly or partly) outside the scope they perceive for the SM, or perhaps outside the scope of the traits/conditions coming to mind as disability-relevant.

Third, not all people who use and endorse the ‘strong’ SM may understand theoretical distinctions between the ‘strong’ and ‘weak’ SM, or these distinctions may seem academic and unimportant in their daily lives. They may be advocating for societal reforms without intending to deny the importance of biological and cognitive differences between people. If so, ‘strong’ SM rhetoric could risk misrepresenting its adherents’ actual views on concrete issues.

Supporting the third interpretation, ‘strong’ SM supporters were, after correction, statistically indistinguishable from ‘weak’ supporters when compared on various items and subscales indexing support for individually versus societally oriented interventions, perhaps implying that their ‘strong’ SM rhetoric masks ‘weak’ SM views. Due to the small number of ‘weak’ supporters, this result should be treated with caution – but these results do at least seem to suggest that any possible difference in views would be of modest magnitude. Thus, it seems clear that ‘strong’ SM rhetoric is something of a distraction, and that in practice, most of its adherents in the autism community remain open to individually focused interventions and supports, used appropriately and judiciously.

That said, one should not conclude this means the SM is irrelevant/facile. Efforts to develop and study autism interventions have overwhelmingly focused on changing autistic individuals, rather than society (Bottema-Beutel et al., 2018; Broderick & Roscigno, 2021; Scott et al., 2019), and currently, autism is diagnosed within a framework that assumes disability/impairment is rooted in the person, largely ignoring context (American Psychiatric Association, 2013); the SM is well-positioned to critique these realities. Moreover, SM supporters staunchly opposed normalization goals from our adapted AAIS. Thus, even a ‘weak’ SM can be transformational.

Understandings of neurodiversity and the NDM

This study also explored qualitative views of neurodiversity/the NDM and quantitative correlates of NDM support/identification. As we predicted in our first hypothesis, NDM support was associated with greater opposition to normalization intervention goals, but not endorsement of

well-being intervention goals. However, contrary to our hypothesis, greater NDM identification was related to decreased support for adaptive skill intervention goals (we had expected no relationship). However, NDM supporters in this study generally reported some support for these adaptive skill goals, arguably in keeping with our hypothesis’ spirit. Participants simply became less enthusiastic about them as their support for the NDM increased. As several adaptive skill scale items concerned social/interpersonal challenges, this might relate to insightful neurodiversity-aligned critiques of social skills training (Autistic Self Advocacy Network, 2021; Bottema-Beutel et al., 2016, 2018).

NDM support was positively associated with support for societal reform and building supportive environments. It was also associated with support for making the world better for people with anxiety, depression and epilepsy – yet only slightly associated with scepticism towards efforts to cure these conditions – emphasizing that pursuit of cure and pursuit of societal reform can, if appropriate, be pursued simultaneously (see Baker, 2011). NDM support was associated with enthusiasm for promoting communication through any effective method rather than prioritizing verbal speech (consistent with Schuck et al., 2022) and with support for autistic leadership and consultancy in intervention decision-making, and reduced support for leadership from parents and professionals.

Few quantitative studies compare experiences of autistic and non-autistic parents, and to our knowledge, this is the first to directly compare views on important political questions such as acceptability of intervention goals and views on who should make decisions in interventions. Autistic parents/caregivers were considerably less supportive of parent leadership in intervention development than non-autistic parents/caregivers. This may reflect advocacy-oriented approaches to parenting among autistic parents/caregivers, as they were also more supportive of the SM and the NDM than their non-autistic counterparts. Autistic parents’ support for child autonomy over parent leadership may also help address concerns that autistic/neurodiversity advocacy leads to ‘parent-blaming’ (Lutz, 2020; Mitchell, 2019). While ‘autism parents’ often do face immense stress and make sacrifices (Hayes & Watson, 2013; Hellman, 2022; Naughton, 2021), these data suggest neurodiversity advocates’ concerns about child autonomy and parents’ power are shared by autistic people who are autism parents themselves (see Crane et al., 2021; Dugdale et al., 2021; Hwang & Heslop, 2023). (Predominantly non-autistic) family/carer perspectives and agency have generally been dominant over autistic people’s in-media portrayals (Holton, 2013; Karaminis et al., 2023; Lewin & Akhtar, 2021); there remains room to address this neglect without dismissing caregivers’ perspectives.

Open-ended questions provided intriguing information regarding conceptualizations of neurodiversity and the

首先，尽管最终将社会视为责任方，并渴望一种社会解决方案，‘强’ SM倡导者可能对聚焦个体的干预持开放态度，至少在某些情境下如此，因为改革社会存在困难。

其次，参与者可能仅在涉及特定的残障或特征时才支持‘强’ SM观点 (Ne’ eman与Pellicano, 2022)，而适应技能、抑郁和癫痫（完全或部分）超出了他们认为SM适用的范围，或者可能超出了他们心目中与残障相关的特质/状况的范围。

第三，并非所有使用并赞同‘强’ SM的人都理解‘强’ SM与‘弱’ SM之间的理论区别，或者这些区别在他们日常生活中可能显得学术化且不重要。他们可能在倡导社会改革的同时，并非意图否认人与人之间生物和认知差异的重要性。如果是这样，‘强’ SM的修辞可能存在误传其支持者在具体问题上实际观点的风险。

支持第三种解释的是，经过校正后，‘强’ SM支持者在各种衡量支持个体导向与社会导向干预的项目和子量表上，与‘弱’支持者在统计上无法区分，这可能意味着他们的‘强’ SM修辞掩盖了‘弱’ SM观点。由于‘弱’支持者数量较少，这一结果应谨慎对待——但这些结果至少似乎表明，观点上任何可能的差异幅度都将是适度的。因此，显然‘强’ SM修辞某种程度上是一种干扰，在实践中，孤独谱系社群中大多数其支持者仍然对个体聚焦的干预和支持持开放态度，只要这些干预和支持被恰当且审慎地使用。

话虽如此，我们不应就此认为社会模型 (SM) 无关紧要或流于肤浅。开发和研究孤独症干预的努力，绝大多数都聚焦于改变孤独谱系个体，而非改变社会 (Bottema-Beutel等, 2018; Broderick与Roscigno, 2021; Scott等, 2019)；并且，目前孤独症的诊断框架仍假定残障/障碍根植于个人，很大程度上忽略了环境因素 (美国精神医学学会, 2013)；社会模型 (SM) 正适合批判这些现实。此外，社会模型 (SM) 的支持者坚决反对我们改编的AAIS中的正常化目标。因此，即使是一个‘弱’版本的社会模型 (SM) 也可能具有变革性。

对神经多样性 (ND) 与神经多样性模型 (NDM) 的理解

本研究还探讨了对神经多样性/神经多样性模型的定性看法，以及神经多样性模型支持/认同的定量关联。正如我们在第一个假设中所预测的，对神经多样性模型的支持与更反对正常化干预目标相关，但并不支持

幸福感干预目标。然而，与我们的假设相反，更强的神经多样性模型认同与对适应技能干预目标的支持度降低相关（我们原本预期没有关联）。不过，本研究中的神经多样性模型支持者总体上对这些适应技能目标仍表现出一定程度的支持，这可以说符合我们假设的精神。参与者只是随着对神经多样性模型的支持增加，对这些目标的热情有所降低。由于适应技能量表中有多项涉及社交/人际挑战，这可能与神经多样性视角下对社交技能训练富有洞察力的批判有关（孤独谱系自我倡导网络, 2021; Bottema-Beutel等, 2016, 2018）。

对神经多样性模型 (NDM) 的支持与社会改革及构建支持性环境的支持度呈正相关。它也与支持为焦虑、抑郁和癫痫患者创造更美好世界相关——但仅与对治愈这些状况的努力持怀疑态度略微相关——这强调，在适当情况下，追求治愈与追求社会改革可以同时进行（参见Baker, 2011）。对神经多样性模型 (NDM) 的支持与热衷于通过任何有效方法促进沟通（而非优先考虑口语表达）相关（这与Schuck等2022年的研究一致），也与支持孤独谱系群体领导及在干预决策中提供咨询相关，同时降低了对由家长和专业人员主导的支持。

很少有定量研究比较孤独谱系与非孤独谱系家长的经历，据我们所知，这是首个直接比较双方在重要政治问题上观点的研究，例如干预目标的可接受性以及对于干预中应由谁决策的看法。孤独谱系父母/照护者对于干预发展中家长主导权的支持程度远低于非孤独谱系父母/照护者。这可能反映了孤独谱系父母/照护者在育儿中倾向于倡导导向的方法，因为他们对社会模型和神经多样性模型的支持度也高于非孤独谱系对照群体。孤独谱系家长对儿童自主权而非家长主导权的支持，也可能有助于缓解关于孤独谱系/神经多样性倡导会导致‘家长指责’的担忧 (Lutz, 2020; Mitchell, 2019)。虽然‘孤独谱系儿童家长’确实常常面临巨大压力并做出牺牲 (Hayes & Watson, 2013; Hellman, 2022; Naughton, 2021)，但这些数据表明，神经多样性倡导者对儿童自主权和家长权力的关切，也得到了身为孤独谱系儿童家长的孤独谱系群体自身的认同（参见Crane et al., 2021; Dugdale et al., 2021; Hwang & Heslop, 2023）。在媒体 portrayal 中，（主要由非孤独谱系构成的）家庭/照护者视角和能动性通常主导了孤独谱系群体的声音 (Holton, 2013; Karaminis et al., 2023; Lewin & Akhtar, 2021)；在不忽视照护者视角的前提下，仍有空间来弥补这种忽视。

开放式问题提供了关于神经多样性概念化的有趣信息，以及

NDM. Although most participants’ definitions of ‘neurodiversity’ framed it as a descriptive term referring to the neurocognitive diversity of humanity, value-laden/prescriptive understandings were also common, as suggested by Ne’eman and Pellicano (2022). This is consistent with the term’s history of being used to suggest that neurological diversity can be neutral or positive (Blume, 1998; Langdon, in Dekker, 2023; Singer, 2016).

Participants usually understood neurodiversity as neurological/biological, similarly to prior research on understandings of autism’s origins (Gillespie-Lynch et al., 2017; Kapp et al., 2013). This may seem consistent with assumptions by some MM adherents that autism has knowable biological markers (Parellada et al., 2023; Shen et al., 2019). However, the high levels of SM and NDM support observed in this study highlight how biological views of the origin (‘etiology’) of human neurocognitive diversity need not prevent people from recognizing societal factors that can contribute to challenges neurodivergent people face. Moreover, in a new measure of neurodiversity beliefs, items about autism’s permanence and neurobiological ‘hard-wiring’ converge with an item rejecting the possibility of curing autism (VanDaalen et al., 2024): apparently, some essentialist biological understandings of neurocognitive differences can motivate rejection of the traditional MM’s cure/prevention agenda (see also Ellis, 2023). However, autism is biologically and causally (‘etiologically’) heterogeneous (Havdahl et al., 2021; Myers et al., 2020). Thus, some NDM leaders and scholars reject biological essentialism and emphasize the social construction of many neurotype categories, including autism (e.g. see Chapman, 2020b; Milton in Milton & Timimi, 2016).

Many autistic/autism community participants’ definitions of neurodiversity referred to cognitive/perceptual aspects, which may help to avoid this tension within biological focuses. Indeed, an interest in cognition has helped autistic people develop descriptive cognitive frameworks to help make sense of their experiences (see, e.g. Buckle et al., 2021; Dekker, 1999; Milton & Timimi, 2016; Murray et al., 2005).

In contrast to this emphasis on biology and cognition, participants’ answers seldom mentioned behaviour, though it is used to diagnose autism, attention-deficit hyperactivity disorder (ADHD), and other neurodivergent categories (American Psychiatric Association, 2013). This is consistent with autistic people’s tendency to define autism in an internal way (Gillespie-Lynch et al., 2017).

Autistic and non-autistic people and ‘neurodiversity lite’

In our second hypothesis, we predicted that autistic neurodiversity proponents would focus more on environmental and societal barriers, and support for interventions capable of changing them, than non-autistic

neurodiversity proponents. Admittedly, when prompted, autistic and non-autistic participants both endorsed societal reform and supportive environment-building intervention goals; their quantitative ratings did not differ. Yet in exploratory comparisons of autistic and non-autistic participants’ free-text descriptions of the NDM, autistic participants spontaneously mentioned efforts to reform society and promote neurodivergent thriving more often, while non-autistic participants spontaneously appeared more focused on the NDM’s celebration of diversity. This is consistent with concerns that some non-autistic people are (perhaps unknowingly) co-opting the NDM by advancing ‘neurodiversity lite’ ideas (Neumeier, 2018).

Also consistent with that concern are findings suggesting that non-autistic people were – even after accounting for neurodiversity support – less sceptical of various intervention goals, including normalization- and adaptive skill-oriented interventions. Thus, some non-autistic neurodiversity proponents may not have fully appreciated concerns about the harms arising from efforts to suppress autistic traits (see, e.g. Anderson, 2023; McGill & Robinson, 2021).

At the same time, these differences should not be over-emphasized. Overall, our data revealed remarkable consensus among participants in this sample, and the overlap between autistic and non-autistic participants was extensive. Even if some degree of co-option of neurodiversity ideas could be reflected in our data, our data overall clearly depict meaningful agreement and allyship between autistic and non-autistic advocates.

Limitations

Although this mixed-methods study offers a rich and nuanced examination of key issues in autistic and autism communities, it is not without limitations. The study describes views and conceptualizations of neurodiversity and models of disability in a predominantly White, well-educated sample of autism community members, mostly from wealthy Anglophone countries. Different ideas are being developed in other cultural contexts (Cheng et al., 2023; Kapp, 2011; Ngwagwa, 2022) and more intersectional and cross-cultural analysis of the NDM is greatly needed. Moreover, language is not static, and NDM advocates – especially neurodivergent advocates – will continue to advance new ideas. Thus, these results are not a definitive idea of what the NDM is or could be.

This study’s autistic sample included few individuals with higher support needs: for example, only seven autistic people with intellectual disabilities and five nonspeaking autistic people. Ironically for a study about the NDM and SM, our efforts to collect highly rich, nuanced data reduced the accessibility of the study. Obtaining perspectives from across the diversity of autism is a crucial direction for further research.

神经多样性模型。尽管大多数参与者对‘神经多样性’的定义将其视为一个描述性术语，指代人类神经认知的多样性，但带有价值判断/规定性的理解也很常见，正如Ne’eman和Pellicano（2022）所指出的。这与该术语的历史用法一致，即被用来暗示神经系统的多样性可以是中性或积极的（Blume, 1998; Langdon, in Dekker, 2023; Singer, 2016）。

参与者通常将神经多样性理解为神经/生物学层面的，这与先前关于孤独谱系起源认知研究类似（Gillespie-Lynch等，2017；Kapp等，2013）。这可能看似符合某些医学模型支持者的假设，即孤独谱系存在可识别的生物学标记（Parellada等，2023；Shen等，2019）。然而，本研究观察到的高水平社会模型和神经多样性模型支持，突显了人类神经认知差异起源（‘病因学’）的生物学观点，并不妨碍人们认识到可能导致神经殊异群体面临挑战的社会因素。此外，在一项新的神经多样性信念测量中，关于孤独谱系的持久性和神经生物学‘硬连接’的条目，与一项拒绝治愈孤独谱系可能性的条目趋于一致（VanDaalen等，2024）：显然，一些对神经认知差异的本质主义生物学理解，可以促使人们拒绝传统医学模型的治愈/预防议程（另见Ellis，2023）。然而，孤独谱系在生物学和因果（‘病因学’）上是异质的（Havdahl等，2021；Myers等，2020）。因此，一些神经多样性模型领导者和学者拒绝生物学本质主义，并强调包括孤独谱系在内的许多神经类型类别的社会建构性（例如参见Chapman，2020b；Milton在Milton & Timimi，2016中）。

许多孤独谱系/孤独谱系社群参与者对神经多样性的定义涉及认知/感知层面，这可能有助于避免在生物学焦点内产生这种张力。确实，对认知的关注帮助孤独谱系群体发展出描述性的认知框架，以助理解自身经验（参见，例如Buckle等人，2021；Dekker，1999；Milton与Timimi，2016；Murray等人，2005）。

与这种对生物学和认知的强调形成对比的是，参与者的回答很少提及行为，尽管行为被用于诊断孤独谱系、注意缺陷多动障碍（ADHD）及其他神经殊异类别（美国精神医学学会，2013）。这与孤独谱系群体倾向于以内在方式定义孤独谱系的特点一致（Gillespie-Lynch等人，2017）。

孤独谱系与非孤独谱系群体及‘神经多样性轻量版’

在我们的第二个假设中，我们预测孤独谱系神经多样性支持者会比非孤独谱系群体更关注环境和社会障碍，以及支持能够改变这些障碍的干预措施。

神经多样性支持者。诚然，在被提示时，孤独谱系和非孤独谱系参与者都认可社会改革和支持性环境建设的干预目标；他们的量化评分并无差异。然而，在对孤独谱系和非孤独谱系参与者关于神经多样性模型的自由文本描述进行探索性比较时，孤独谱系参与者自发提及改革社会和促进神经殊异繁荣的努力更为频繁，而非孤独谱系参与者自发地似乎更关注神经多样性模型对多样性的颂扬。这与一些非孤独谱系群体（或许是无意识地）通过推进‘神经多样性轻量版’理念（Neumeier, 2018）来挪用神经多样性模型的担忧是一致的。

与这种担忧相一致的是，有研究表明，即使在考虑了神经多样性支持之后，非孤独谱系群体对各种干预目标——包括以正常化和适应技能为导向的干预——的怀疑程度较低。因此，一些非孤独谱系的神经多样性支持者可能并未充分认识到压制孤独谱系特质所带来的危害担忧（参见，例如Anderson，2023；McGill与Robinson，2021）。

同时，这些差异也不应被过分强调。总体而言，我们的数据显示了本样本参与者之间存在显著的共识，且孤独谱系与非孤独谱系参与者之间的重叠范围很广。即使我们的数据可能反映出神经多样性理念在某种程度上被借用，但我们的数据总体上清晰地描绘了孤独谱系与非孤独谱系倡导者之间有意义的共识与同盟关系。

局限性

尽管这项混合方法研究对孤独谱系及孤独症社群中的关键议题进行了丰富而细致的考察，但它并非没有局限性。本研究描述了一个以白人、高学历为主的孤独谱系社群成员样本（大多来自富裕的英语国家）对神经多样性及残疾模式的看法和概念化。不同的理念正在其他文化背景下发展（Cheng et al., 2023; Kapp, 2011; Ngwagwa, 2022），对神经多样性模型进行更多交叉性和跨文化的分析是迫切需要的。此外，语言并非静止不变，神经多样性模型的倡导者——尤其是神经殊异倡导者——将继续推进新的理念。因此，这些结果并非对神经多样性模型是什么或可能是什么的最终定论。

本研究的孤独谱系样本中，包含较高支持需求的个体较少：例如，仅有七名患有智力障碍的孤独谱系群体成员和五名无口语孤独谱系群体成员。讽刺的是，对于一个关于神经多样性模型和社会模型的研究，我们为收集高度丰富、细致数据所做的努力反而降低了研究的可及性。获取来自孤独谱系多样性各方的观点，是未来研究的关键方向。

Finally, although we attempted to direct recruitment invitations to organizations and communities from across the spectrum of ‘autism politics’ opinion, neurodiversity proponents appeared more likely to participate. Thus, while the data depict consensus in favour of the NDM and societal reform, and strong opposition to normalization and the MM (which could reflect growing awareness of the NDM: see Kapp et al., 2013), this may not be representative of the autism community overall.

Future directions

To address these limitations, one direction for future research could be to examine whether this study’s results hold true in samples recruited using other approaches: for example, sampling online databases such as Prolific, recruiting in-person in local communities, engaging marginalized populations within wealthy Anglophone countries (e.g. indigenous nations, socioeconomically deprived locales) or focusing recruitment on other countries. New studies would also allow our interpretations of this study data to be turned into explicit propositions, so that one could investigate the degree to which community members agree with the interpretations put forth in this article.

Another important direction for future research will be clarifying the acceptability of specific discourses and practices. For example, given the large number of socially relevant items on the adaptive skills scale, these findings suggest ‘teaching adaptive skills’ such as social-communication skills may be acceptable in principle, especially when done mutually with interaction partners (Milton, 2012). Yet our results and prior investigations of the acceptability of social skills training (Bottema-Beutel et al., 2016) and Pivotal Response Treatment (Schuck et al., 2022) show that this by no means guarantees that key practices will be endorsed by community members (e.g. concerns about normalization and lack of child autonomy; Autistic Self Advocacy Network, 2021). Experiences of misunderstanding, stigma and bias from professionals and educators (e.g. see Brownlow et al., 2021; Darazsdi & Bialka, 2023; McGill & Robinson, 2021) could also adversely affect the actual acceptability of supports. Furthermore, because of the paramount importance of customizing supports to the needs and perspectives of individuals (and, as appropriate, their supporters), tools should be developed to help individuals (and, as appropriate, representatives such as family members) make meaningful choices about supports, goals and practices in their own contexts.

Conclusion

Notwithstanding limitations and associated calls for follow-up research, we believe our results provide some of

the most comprehensive available evidence today regarding what neurodiversity, the NDM, and the SM of disability mean to supporters in the autistic and autism communities. Those with concerns about the NDM or SM should read these results carefully and ensure that any critiques they offer describe NDM and SM advocates’ views accurately. If debates remain free of mischaracterizations or false stereotypes, it will be easier for NDM supporters and their critics to make progress in discussions about genuine disagreements. In particular, this study suggests that SM rhetoric masks the more nuanced views most of its adherents have on concrete questions regarding intervention goal appropriateness. It indicates that neurodiversity supporters denounce normalization, and can be open to curing co-occurring conditions and teaching adaptive skills; future research and debate could continue to explore how the latter could be pursued in neurodiversity-affirmative and socially valid ways.

Furthermore, although our study was not designed to determine how common neurodiversity-affirming views are within the autistic and autism communities, participants’ clear enthusiasm for the NDM does suggest that improving access to neurodiversity-affirmative supports is necessary to serve increasing numbers of neurodiversity-supporting community members. In addition to confirming that self-identified neurodiversity supporters are generally opposed to interventions aiming to suppress and normalize autistic traits and behaviours, this study directs attention towards the importance of power dynamics, with neurodiversity supporters desiring additional prioritization of autistic leadership and involvement. Thus, interventions and supports reflecting these preferences must be made available to neurodiversity-supporting individuals and families. Moreover, though there were some differences between autistic and non-autistic respondents (including autistic and non-autistic parents), there were relatively few/modest differences overall: all groups expressed strong, striking desires for societal reform and efforts to make the world more inclusive of autistic and neurodivergent people.

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Author Contributions

P.D., S.K.K., K.G.-L. and S.M.R. designed this study. Recruitment was conducted by S.K.K., P.D. and K.G.-L. P.D. and D.S.C. coded and removed invalid data. P.D., A.N.G., E.K. and S.K.K. coded data from the open-ended questions presented in this report. Quantitative analysis was conducted by P.D., with contributions from L.H.H. This article was drafted by P.D. and edited and approved by all authors.

最后，尽管我们试图将招募邀请定向发送给涵盖‘孤独谱系政治’观点光谱的组织和社群，但神经多样性倡导者似乎更有可能参与。因此，虽然数据显示了支持神经多样性模型和社会改革的共识，以及对正常化和医学模型的强烈反对（这可能反映了对神经多样性模型日益增长的认知：参见Kapp et al., 2013），但这可能并不代表整个孤独谱系社群的普遍观点。

未来方向

为应对这些局限，未来研究的一个方向可以是检验本研究的结果在使用其他方法招募的样本中是否同样成立：例如，通过在线数据库（如Prolific）抽样、在当地社区进行线下招募、吸纳富裕英语国家中的边缘化群体（例如原住民群体、社会经济贫困地区），或将招募重点放在其他国家。新的研究也将允许我们将对本研究数据的解读转化为明确的命题，从而可以调查社区成员对本文提出的解读的认同程度。

未来研究的另一个重要方向将是澄清特定话语和实践的可接受性。例如，鉴于适应技能量表上有大量与社会相关的项目，这些发现表明“教授适应技能”（如社交沟通技能）原则上可能是可接受的，尤其是在与互动伙伴共同进行时（Milton, 2012）。然而，我们的结果以及先前对社交技能训练（Bottema-Beutel等, 2016）和关键反应训练（Schuck等, 2022）可接受性的调查表明，这绝不意味着关键实践会得到社区成员的认可（例如，对正常化和缺乏儿童自主权的担忧；孤独谱系自我倡导网络, 2021）。来自专业人员和教育工作者误解、污名和偏见经历（例如，参见Brownlow等, 2021; Darazsdi与Bialka, 2023; McGill与Robinson, 2021）也可能对支持的实际可接受性产生负面影响。此外，由于根据个体（以及酌情包括其支持者）的需求和观点定制支持至关重要，应开发工具来帮助个体（以及酌情包括其代表，如家庭成员）在其自身情境中就对支持、目标和实践做出有意义的选择。

结论

尽管存在局限性并伴随后续研究的呼吁，我们相信我们的结果提供了

当前关于神经多样性、神经多样性模型以及残疾的社会模型对孤独谱系及孤独症社群中的支持者意味着什么的最全面的现有证据。对神经多样性模型或社会模型有疑虑者应仔细阅读这些结果，并确保他们提出的任何批评能准确描述神经多样性模型和社会模型倡导者的观点。如果辩论能避免错误描述或虚假刻板印象，神经多样性支持者与其批评者就更容易在关于真正分歧的讨论中取得进展。特别是，本研究表明，社会模型的修辞掩盖了其大多数追随者在干预目标适当性等具体问题上更为细致的观点。它指出，神经多样性支持者谴责正常化，并且可能对治疗共病状况和教授适应技能持开放态度；未来的研究和辩论可以继续探索如何以肯定神经多样性且社会认可的方式追求后者。

此外，尽管我们的研究并非旨在确定神经多样性肯定观点在孤独谱系及孤独症社群中有多普遍，但参与者对神经多样性模型的明确热情确实表明，改善对神经多样性肯定支持的获取途径是必要的，以满足日益增多的支持神经多样性的社群成员的需求。除了证实自我认同的神经多样性支持者普遍反对旨在抑制和正常化孤独谱系特质与行为的干预措施外，本研究还引导人们关注权力动态的重要性，神经多样性支持者希望更多地优先考虑孤独谱系群体领导与参与。因此，必须向支持神经多样性的个体和家庭提供反映这些偏好的干预与支持。此外，尽管孤独谱系与非孤独谱系受访者（包括孤独谱系家长与非孤独谱系家长）之间存在一些差异，但总体差异相对较少/有限：所有群体都表达了强烈且显著的愿望，希望进行社会改革，并努力使世界对孤独谱系及神经多样性人群更具包容性。

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Positionality Statement

The authors identify as supporters or allies of the neurodiversity movement.

P. D. is a White Canadian autistic autism researcher, a neurodiversity advocate, and a supporter of social-ecological/interactionist models of disability. P.D.’s main research focus is neurodivergent sensory experience, his background is in developmental and cognitive science, and he is broadly interested in many factors relevant to neurodivergent people’s well-being. ANG is a White, cisgender neurodivergent and disabled woman from the United States, a disability and neurodiversity advocate and autistic autism researcher. She also has cerebral palsy and mental health disabilities, which she also sees as important identities within the neurodiversity movement. She has a Master’s degree in neuroscience but is currently working in many areas of autism research, including neurodiversity-affirming interventions and supports, mental health in autistic people and autistic identity. Finally, she recognizes that her position is not reflective of all forms of oppression and experience, especially those faced by racial and ethnic minorities, those with LGBTQ+ identities, and those with disabilities that extend beyond her own experience.

S. K.K. is a White cisgender man, American and permanent UK resident, disability rights and neurodiversity activist, developmental psychologist and autistic autism researcher. S.K.K.’s research broadly considers the nature or meaning of autism and neurodiversity as well as means of supporting autistic and other neurodivergent people.

E. K. is a White, autistic autism researcher from the United States who supports the neurodiversity movement. She advocates for neurodiversity-affirming practices in interventions, education and research. Her autistic identity gives her some level of understanding of oppression (e.g. ableism), but recognizes that being a part of the dominant racial group, her privilege is a large part of her positionality. She tries to learn and listen to the BIPOC autistic community and is on a continuous journey of allyship and cultural humility.

D. S.C. is a Taiwanese-American educator from the United States with a background in autism research. D.S.C.’s work with children of diverse backgrounds within various educational contexts has guided his interest in researching the experience of neurodivergent individuals. He works to continue building a perspective of allyship and cultural responsiveness in both research and education.

SMR is a White, LatinX researcher from the United States. Her research investigates questions about how underlying brain

activity and behaviour supports the development of skills (e.g. attention, visual perception, sensory processing and emotion regulation) necessary for adaptive cognition and social-emotional well-being. In addition to this scientific work, she is also committed to championing the tenets of neurodiversity and advocacy to the public and the academic community.

K. G.-L. is a White, bisexual woman from the United States. She was very briefly in special education in kindergarten, purportedly due to skipping in an unusual way, before being moved into a gifted group. She experiences some executive functioning and social differences but has not been diagnosed with a disability. She has many neurodivergent family members and close friends.

ORCID iDs

Patrick Dwyer  <https://orcid.org/0000-0003-1779-5252>

Steven K. Kapp  <https://orcid.org/0000-0002-4440-1688>

Kristen Gillespie-Lynch  <https://orcid.org/0000-0001-7292-3945>

Supplemental material

Supplemental material for this article is available online.

Notes

1. As discussed in the study by Dwyer (2022), the term ‘paradigm’, as introduced by Kuhn (1962/2012), does not merely mean ‘theoretical framework’. Rather, in Kuhnian philosophy of science, paradigms offer exemplars suggesting approaches that should be taken towards experimental design, writing and so on. Analogously, neurodiversity approaches suggest how human neurocognitive diversity should be approached by society.
2. This nomenclature reflects the SM’s origins in physical disability advocacy. In the neurodiversity context, where neurodivergent people’s differences can sometimes be strengths in suitable contexts, terms such as ‘differences’ or ‘neurodivergence’ might be more appropriate.
3. Note that Shakespeare is here cited as a disability advocate, not a neurodiversity advocate; Shakespeare (2014, pp. 152–153) expressed concerns about the neurodiversity concept’s ability to deal with heterogeneity of autism.
4. Briefly, among those aware of these models, there was strong support for the NDM and SM, but not the MM (see Supplementary Figure C.1; Supplementary Table C.2). Participants generally strongly supported well-being and societal reform intervention goals, somewhat more tepidly supported adaptive skill and supportive environment goals, and opposed normalization goals. According to Mann–Whitney tests conducted for this article, autistic people were slightly more supportive of the NDM, somewhat more enthusiastic about supportive environment goals, slightly less supportive of well-being intervention goals (likely due to scepticism as to how those goals would be pursued), somewhat less supportive of the MM, and much less supportive of normalization and adaptive skill intervention goals (see Supplementary Table C.2).
5. Participants had opportunities to provide additional thoughts as they navigated the survey. A representative comment from an autistic parent regarding well-being goals was, ‘Who is defining these goals? Who is setting the definition

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立场声明

作者们自认为是神经多样性运动的支持者或盟友。

P.D. 是一位加拿大白人孤独谱系研究者、神经多样性倡导者，也是社会生态/互动主义残疾模式的支持者。P.D. 的主要研究重点是神经殊异群体的感官体验，其背景是发展心理学和认知科学，并且他对与神经殊异群体幸福感相关的诸多因素有着广泛的兴趣。

ANG 是一位来自美国的白人、顺性别神经殊异及残障女性，也是一位残障与神经多样性倡导者及孤独谱系研究者。她还患有脑瘫和心理健康障碍，她也将这些视为神经多样性运动中的重要身份认同。她拥有神经科学硕士学位，但目前从事孤独谱系研究的多个领域，包括神经多样性肯定的干预和支持、孤独谱系群体心理健康以及孤独谱系身份认同。最后，她认识到她的立场并不能反映所有形式的压迫和经历，尤其是种族和少数族裔群体、拥有 LGBTQ+ 身份认同的人以及那些残疾经历超出她自身经验的人所面临的境况。

S.K.K. 是一位白人顺性别男性，美国人兼英国永久居民，残障权利与神经多样性运动倡导者，发展心理学家及孤独谱系研究者。S.K.K. 的研究广泛探讨孤独谱系与神经多样性的本质或意义，以及支持孤独谱系及其他神经殊异群体的方法。

E.K. 是一位来自美国的白人孤独谱系研究者，她支持神经多样性运动。她倡导在干预、教育程度和研究领域采用神经多样性肯定性实践。她的孤独谱系身份认同让她对压迫（例如残障歧视）有一定程度的理解，但也认识到作为主导种族群体的一员，她的特权是其立场的重要组成部分。她努力学习和倾听BIPOC孤独谱系社群的声音，并持续走在同盟关系与文化谦逊的旅程上。

D.S.C. 是一位来自美国的台裔美国人教育工作者，拥有孤独谱系研究背景。D.S.C. 在不同教育背景下与多元背景儿童的工作经历，引导了他对研究神经殊异个体体验的兴趣。他致力于在研究和教育程度领域持续构建同盟关系与文化响应性的视角。

SMR是一位来自美国的白人拉丁裔研究者。她的研究探讨了关于底层大脑

活动与行为如何支持技能发展（例如注意力、视觉感知、感觉处理和情绪调节）的问题，这些技能对于适应性认知和社会情感健康至关重要。除了这项科学工作，她还致力于向公众和学术界倡导神经多样性的原则与主张。

K. G.-L. 是一位来自美国的白人双性恋女性。她曾在幼儿园短暂接受过特殊教育，据称是因为她以一种不寻常的方式跳跃，之后被转入了天才班。她存在一些执行功能和社交方面的差异，但尚未被诊

断为患有残疾。y. 她有许多神经殊异的家庭成员和亲密朋友。

ORCID iDs

Patrick Dwyer  <https://orcid.org/0000-0003-1779-5252>

Steven K. Kapp  <https://orcid.org/0000-0002-4440-1688>

Kristen Gillespie-Lynch  <https://orcid.org/0000-0001-7292-3945>

补充材料

本文的补充材料可在网上获取。

注释

1. 正如Dwyer（2022）的研究中所讨论的，Kuhn（1962/2012）提出的术语‘范式’并非仅y指‘理论框架’。相反，在库恩科学哲学中，范式提供了范例，提示了在实验设计、写作等方面应采取的方法。类似地，神经多样性方法提示了社会应该如何对待人类的神经认知多样性。
2. 这一命名反映了社会模型起源于身体残障倡导。在神经多样性语境中，神经多样性人群的差异有时在合适的背景下可能是优势，因此诸如‘差异’或‘神经多样性’的术语可能更为恰当。
3. 请注意，此处引用Shakespeare是作为一位残障倡导者，而非神经多样性倡导者；Shakespeare（2014，第152–153）对神经多样性概念处理孤独谱系异质性的能力表达了担忧。
4. 简而言之，在了解这些模型的人群中，对NDM和SM的支持度很高，但对MM则不然（参见补充图C.1；补充表C.2）。参与者普遍强烈支持幸福感和社会改革干预目标，对适应技能和支持性环境目标的支持稍显温和，并反对正常化目标。根据本文进行的曼-惠特尼检验，孤独谱系群体对NDM的支持度略高，对支持性环境目标的热情稍强，对幸福感干预目标的支持度略低（可能源于对如何实现这些目标的疑虑），对MM的支持度稍弱，而对正常化和适应技能干预目标的支持度则大幅降低（参见补充表C.2）。
5. 参与p蚂蚁已经pp机遇p提供额外想法ghts正如他们在填写问卷时所表达的。一位代表性的评论一位孤独谱系家长关于福祉目标的疑问是：‘谁在定义这些目标？谁在设定生活质量的定义

of quality of life? Who is setting the definition of good? Or Improving? Or harmful?” Similar concerns were raised by some autistic people who are not parents: for example, ‘Hopefully by “reducing depression” this means improving self-esteem and quality of life not just medicating’.

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？ 谁在设定“好”的定义？ 或者“改善”？ 或者“有害”？ 一些非家长的孤独谱系群体也提出了类似的担忧： 例如， ‘希望“减少抑郁” 指的是提升自尊和生活质量， 而不仅仅是药物治疗’ 。

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